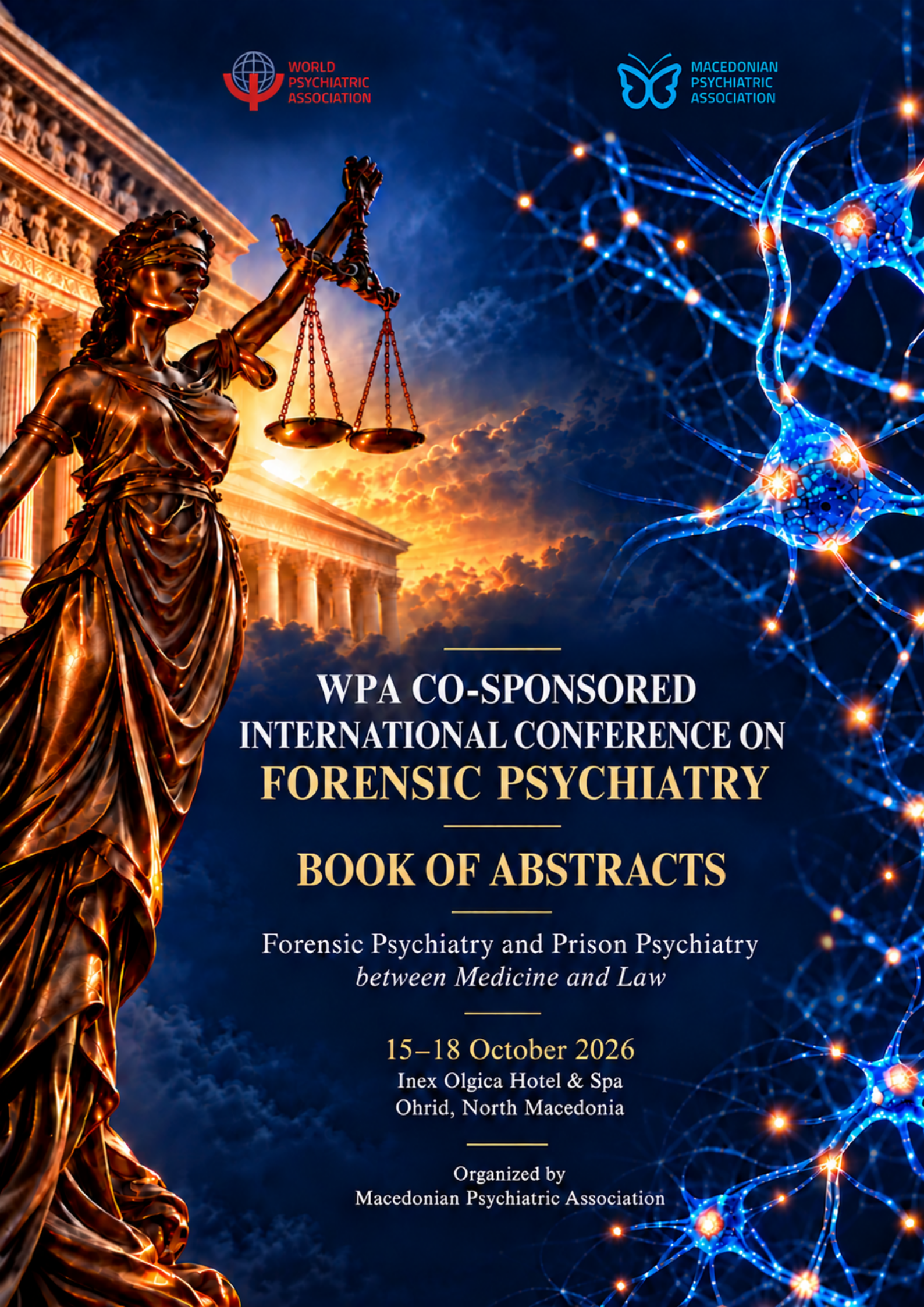


WPA CO-SPONSORED INTERNATIONAL CONFERENCE ON FORENSIC PSYCHIATRY

BOOK OF ABSTRACTS

Forensic Psychiatry and Prison Psychiatry
between Medicine and Law

15–18 October 2026
Inex Olgica Hotel & Spa
Ohrid, North Macedonia

Organized by
Macedonian Psychiatric Association

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Editor

Prof. Antoni Novotni, MD, PhD

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1. Violence Risk Assessment: An Update

Author: Gary Chaimowitz (McMaster University, St. Joseph's Healthcare, Hamilton, Canada)

Violence risk assessment is a central responsibility in forensic psychiatry, with implications for treatment, public safety and individual liberty. This presentation reviews the evolution of assessment from unstructured clinical judgment to actuarial methods and structured professional judgment, and examines contemporary approaches to integrating these methods into clinical practice.

The presentation considers the strengths and limitations of commonly used instruments, including their predictive performance, applicability among populations and ability to inform individual management. It pays particular attention to the distinction between predicting violence and preventing it, interpreting risk categories, and communicating uncertainty. Emerging applications of artificial intelligence and routinely collected clinical data are considered alongside questions of validation, transparency and bias.

A central theme is the contribution of multidisciplinary teams to understanding changes in mental state, behaviour and circumstances. Assessment findings should support an individual formulation that identifies foreseeable scenarios, modifiable factors, protective resources and practical interventions. Ethical considerations include proportionality, cultural context and the consequences of false-positive judgments.

Participants will develop a clearer understanding of contemporary assessment approaches, recognize their limitations, and consider how to translate findings into collaborative treatment and risk-management plans while providing balanced opinions to courts and other decision-makers.

2. Implementation and Evaluation of a Participatory Advisory Board in Forensic Mental Health Research

Author: Völlm Birgit (University of Rostock, Germany)

Background: People with lived experience of forensic mental health care are rarely involved in shaping the research that affects them. Participatory research offers an approach to integrate experiential knowledge on an equal footing alongside scientific and clinical expertise. However, meaningful participation in highly regulated and security-driven forensic settings presents specific challenges.

Methods: In December 2023, the participatory advisory board forensic was established at Rostock University Medical Center to facilitate collaboration between people with lived experience and researchers. The Board meets regularly to discuss research questions, projects and materials and to provide feedback and recommendations across the research process. Its development and implementation were evaluated from the perspectives of people with lived experience and researchers. Structured interviews were conducted with people with lived experience at different stages of the project, complemented by questionnaires on advisory board meetings and project presentations. From the researcher perspective, feedback on recommendations, their implementation and the perceived usefulness of the Board was assessed.

Results: Participation was feasible when supported by clear yet flexible structures, respectful communication, appropriate preparation and support, and researcher commitment. The Board contributed to concrete changes in research practice, including methodological recommendations, adaptations of psychoeducation materials, and e.g. input into the development of ethical guidelines for virtual reality in forensic psychiatry. Lived experience complemented scientific expertise and contributed to the relevance and credibility of research. Challenges included time and resource requirements, institutional constraints, power asymmetries, clarification of roles and influence, and the need to balance safety with openness and flexibility.

Conclusion: The experience of the PART Advisory Board in forensic psychiatry demonstrates that meaningful participation is feasible in forensic mental health research and can influence research processes and decisions. Sustainable involvement requires appropriate structures, resources, support and institutional commitment. Since May 2025, an overarching PART Advisory Board has been established as an independent structure at Rostock University Medical Center, providing a model for sustainable and potentially transferable participation. Participation should not be regarded as an add-on, but as a core quality criterion of patient-centred forensic mental health research.

3. From Norms To Neurons: Legal Capacity And Neurocognitive Disorders - What The Law Asks - And What The Brain Must Deliver

Authors: Antoni Novotni (University Clinic of Psychiatry, Faculty of Medicine, Ss Cyril and Methodius, Skopje, North Macedonia, Institute for Alzheimer's Disease and Neuroscience, Skopje, North Macedonia), Gabriela Novotni (University Clinic of Neurology, Faculty of Medicine, Ss Cyril and Methodius, Skopje, North Macedonia, Institute for Alzheimer's Disease and Neuroscience, Skopje, North Macedonia)

Decision-making capacity sits at the crossroads of neuroscience, law, and ethics. This talk examines the intersection between neurocognitive function and legal capacity, with a particular focus on testamentary capacity in individuals with neurocognitive disorders. Grounded in contemporary neuroscience of volition and agency, it outlines how neural systems supporting intention, self-awareness, executive control, and social cognition enable autonomous decision-making. The talk differentiates global legal capacity from task-specific and time-specific decision-making capacity and revisits the *Banks v. Goodfellow* criteria, linking each legal requirement to the cognitive operations underlying understanding, appreciation, reasoning, and the communication of a stable choice.

The presentation reviews how major neurocognitive disorders - especially Alzheimer's disease, frontotemporal dementias, primary progressive aphasia, and dementia with Lewy bodies affect the abilities relevant to making a will. It emphasizes that a diagnosis of dementia does not establish incapacity, rather, assessment must consider preserved cognitive strengths, the nature of the decision, and the contextual demands of the testamentary act. Structured clinical interviews and neuropsychological tools, including the Testamentary Capacity Assessment Tool, can enhance the evidentiary basis available to legal professionals, while courts retain ultimate authority in determining capacity. This talk provides a clear bridge between the law's expectations and the brain's capacities and sets the stage for examining how neurocognitive disorders disrupt these processes in distinct, phenotype - specific ways.

4. Jail Psychiatry in Practice: Balancing Risk, Treatment and Forensic Accountability

Author: Maryana Kravtsenyuk (University of Toronto, Canada)

Jail psychiatry operates at the complex intersection of clinical care, institutional security, legal accountability, and human rights. Individuals entering custody frequently present with acute psychiatric illness, substance use disorders, elevated suicide risk, and significant psychosocial vulnerabilities, often compounded by fragmented healthcare and uncertain legal circumstances. Drawing on practical experience in correctional and forensic psychiatry, this presentation explores the challenges of delivering ethical, evidence-based psychiatric care within custodial environments. Particular attention is given to early clinical assessment, dynamic risk formulation, treatment under institutional constraints, professional independence, and the distinction between therapeutic and forensic responsibilities. The presentation emphasizes that effective jail psychiatry requires more than managing dangerousness; it demands balancing public safety and legal obligations with compassionate treatment, clinical integrity, and respect for human dignity.

5. Special Populations And Contemporary Challenges In Forensic Psychiatry

Authors: Carlos Isaac, Karina Orozco, Emilce Blanc, Maria Grecco (WPA)

Forensic psychiatry faces complex challenges due to shifting societal dynamics and distinct clinical populations. It requires specialized risk assessment protocols, trauma informed clinical frameworks, and structural system reform across international settings.

This presentation integrates different methodologies to analyze specific sub domains. Carlos Isaac examines modern psychopathologic profiles and risk management associated with cannibalism as an extreme forensic global challenge. Karina O. provides a sociocultural evaluation of feminicide in Latin America, emphasizing specialized gender based investigative frameworks. Mariel G. analyzes managing individuals found Not Guilty by Reason of Insanity. Emilce B. reviews the intersection of neurocognitive decline, and legal responsibility within the aging delinquent population.

Preliminary analyses demonstrate that extreme clinical behaviors such as cannibalism require contextualization within severe psychotic decompensation or complex paraphilic disorders rather than standard violent offending models. In Latin America, addressing feminicide demands specialized forensic protocols to navigate entrenched patriarchal dynamics and legal impunity. Regarding NGRI dispositions, a persistent deficit in community based psychiatric resources hinders optimal rehabilitation and safe reintegration. The rising demographic of older adult offenders presents unique clinical challenges.

Contemporary forensic practice must transition toward highly specialized, ethically grounded interventions tailored to unique subpopulations. Standardizing gender-sensitive protocols, expanding post acquittal mental health infrastructure, and adapting forensic assessments to aging populations are critical steps to improving both clinical management and public safety.

Keywords: Forensics, Cannibalism, Feminicide, Insanity, Elderly.

6. Forensic Aspects of Domestic Violence

Author: Slavica Arsova Hadzi-Angjelkovska (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia)

Domestic violence, and intimate partner violence (IPV) in particular, is at once a public-health problem, a cause of psychiatric illness and a legal matter. Worldwide, about one in four ever-partnered women experiences physical or sexual IPV, and 38.6% of murdered women are killed by a partner. Psychological violence and coercive control are as traumatic as physical abuse, and repeated abuse often leads to complex PTSD. Rates and disclosure vary with social norms and laws, as comparisons between Europe, the Arab states and the Americas show. The perinatal period is a special window: one in four pregnant women experiences IPV, which harms both mother and child, yet antenatal care offers the best chance of detection.

In North Macedonia, police reports of domestic violence rose by 23% from 2022 to 2025, while only 2% of women abused by a current partner report it to the police. A retrospective review at the University Clinic of Psychiatry – Skopje (January 2025 – July 2026) identified 180 patients with stress-related (F43) diagnoses linked to domestic violence. Their share among all F43 patients rose from 7.9% to 15.0%, and they spanned all ages. A case from our Day Hospital shows how partner violence reaches the whole family, through complicated grief and secondary victimisation.

Finally, the lecture addresses the psychiatrist's role: (a) therapeutic and (b) forensic-psychiatric. In the therapeutic role, the psychiatrist recognises violence, offers first-line support, treats its psychological consequences and plans the patient's safety. As a court-appointed expert, the psychiatrist assesses the victim's psychological damage and its causal link with the violence. They also evaluate the perpetrator's criminal responsibility, dangerousness and need for court-ordered treatment, and advise the court on the risk of further violence and on protecting children. Careful clinical documentation is the basis of both roles, and the two roles should be kept separate in the same case.

Keywords: domestic violence; intimate partner violence; forensic psychiatry; perinatal period; secondary victimisation; risk assessment.

7. The Forensic-Psychiatric Impact of Alcohol Consumption on Serious Criminal Offenses Against Traffic Safety

Authors: Dimitar Bonevski, Andromahi Naumovska (Psychiatric Hospital Skopje, HC Skopje)

Alcohol consumption remains one of the most significant preventable contributors to serious and fatal road traffic accidents worldwide. This presentation examines the forensic-psychiatric impact of alcohol consumption on serious criminal offenses against traffic safety, with particular emphasis on neuropsychiatric impairment, criminal responsibility, recidivism, and prevention. Alcohol adversely affects executive functions, attention, judgment, decision-making, emotional regulation, coordination, visual perception, and reaction time, substantially increasing the likelihood of dangerous driving and severe traffic outcomes. The presentation also addresses the forensic psychiatric evaluation of intoxicated drivers, including assessment of mental status, intoxication, alcohol-induced psychiatric disorders, criminal responsibility, legal competency, and risk of recidivism. Although voluntary intoxication generally does not exclude criminal responsibility, exceptional circumstances, such as pathological intoxication, alcohol-induced psychotic disorders, or severe cognitive impairment, may require specialized forensic assessment. Particular attention is given to challenges in determining blood alcohol concentration following delayed sampling and to recent developments in judicial practice in North Macedonia. Risk factors for recurrent alcohol-related traffic offenses include alcohol dependence, previous DUI, impulsivity, aggressiveness, antisocial traits, ADHD, and poor insight. Effective prevention requires coordinated medical, psychological, legal, and public-health interventions, combining treatment and behavioral interventions with enforcement, education, and stricter traffic-safety measures.

Keywords: alcohol intoxication; road traffic safety; forensic psychiatry; criminal responsibility; DUI; recidivism; alcohol use disorder; traffic accidents; prevention; North Macedonia.

8. Precision Medicine versus Experience-Based Forensic Expertise

Author: Nensi Manusheva (University Clinic of Psychiatry, Faculty of Medicine, Ss Cyril and Methodius, Skopje, North Macedonia)

Psychiatry nowadays is moving towards new advances in diagnostic systems, neurobiology/neuroscience and artificial intelligence. The rising knowledge in the field of precision medicine and psychiatry are demanding from psychiatrists to think about individual aspects of diagnosis and treatment. Also, these achievements are transforming the way that forensic expertise is done. In cases where objective parameters or biomarkers of different states are present, we must not neglect them.

Forensic psychiatry as an intersection between mental health and justice should be increasingly involved about current neurobiological evidence and data-driven approaches that can complement traditional clinical assessments.

Key words: forensic expertise, precision medicine, experience.

9. Psychiatry, Human Rights and CPT Standards: Ethical and Legal Challenges in Psychiatric Institutions – with a Focus on North Macedonia

Author: Stojan Bajraktarov (University Clinic for Psychiatry, Medical Faculty, University "Ss. Cyril and Methodius", Skopje, North Macedonia)

Psychiatric institutions are among the few settings in which the state may lawfully deprive a person of liberty and impose treatment without consent. This places psychiatry at the intersection of medical ethics, human rights law and criminal justice. The standards of the European Committee for the Prevention of Torture (CPT) – on safeguards for involuntary placement and treatment, restraint and seclusion, living conditions, staffing, complaint mechanisms and independent monitoring – have become the common European benchmark for mental-health systems. The challenges they address are shared across the region: prolonged institutionalisation, the gap between legislation and daily practice, limited community alternatives and the particular vulnerability of forensic patients under compulsory treatment.

North Macedonia is presented as an illustrative case. CPT reports following several visits have addressed the country's psychiatric hospitals and special institutions, while the Law on Mental Health, the designation of the Ombudsman as National Preventive Mechanism under OPCAT and the national deinstitutionalisation strategy mark a decade of reform. The resulting picture – legislative progress alongside persistent implementation gaps – mirrors developments in neighbouring countries.

Compliance with CPT standards cannot be achieved through legal reform alone; it requires a change in institutional culture, investment in community-based care and the active engagement of psychiatrists as guarantors of patients' rights.

10. Anxiety Disorders in Forensic Psychiatry: Assessment, Legal Relevance, and Expert Opinion

Author: Milan Latas (Faculty of Medicine, University of Belgrade, Serbia)

Background: Anxiety disorders are frequently encountered in forensic psychiatric practice and may have significant implications for legal decision-making. Despite their high prevalence, these disorders are often underestimated in forensic evaluations, particularly in relation to criminal responsibility, competency to stand trial, and risk assessment.

Objectives: The aim of this presentation is to examine the clinical characteristics of anxiety disorders within forensic populations and to highlight their relevance in medico-legal assessments. Special emphasis is placed on diagnostic challenges, evaluation of symptom validity, and the interface between psychiatric findings and legal standards.

Methods: The lecture is based on a narrative review of relevant psychiatric and forensic literature, combined with clinical experience from forensic psychiatric assessments. Key diagnostic criteria, commonly used assessment instruments, and legal concepts applicable to anxiety disorders will be discussed.

Results: Anxiety disorders may significantly affect cognitive functioning, emotional regulation, and behavioral control in legally relevant contexts. In forensic evaluations, comorbidity, stress-related symptom exacerbation, and differentiation from malingering represent major challenges. Anxiety disorders may influence expert opinions regarding criminal responsibility, competency to stand trial, and sentencing considerations.

Conclusion: Accurate assessment of anxiety disorders is essential in forensic psychiatric practice. A thorough understanding of their clinical presentation and legal relevance enables forensic psychiatrists to provide balanced, evidence-based expert opinions while maintaining ethical and professional standards.

11. From Neurons to Norms - How Dementia Types Shape Capacity Vulnerabilities

Author: Gabriela Novotni (University Clinic of Neurology, Faculty of Medicine, Ss Cyril and Methodius, Skopje, North Macedonia, Institute for Alzheimer's Disease and Neuroscience-Skopje, North Macedonia)

As the prevalence of dementia continues to increase annually, the decision-making capacity of affected individuals has emerged as a major concern, both within financial institutions, courts and in clinical practice. In my talk I will explore how distinct dementia phenotypes shape vulnerabilities in decision-making, with a particular focus on testamentary capacity. Drawing on clinical cases across typical and atypical Alzheimer's disease, frontotemporal dementia, primary progressive aphasia, and dementia with Lewy bodies, the talk demonstrates why diagnostic labels, and global cognitive screening scores offer an incomplete basis for capacity determinations. Executive and social-cognitive impairments may compromise judgment despite relatively preserved memory, while visuospatial or language impairment may obscure intact reasoning. Cognitive fluctuations as in LBD, further underscore the importance of timing, context, and the specific demands of the testamentary act. I will then outline advanced and assisted decision-making frameworks, including early advance directives and supported decision-making models that preserve autonomy despite cognitive decline. It situates capacity assessment within a human-rights context, addressing ageism, paternalism, and the ethical obligation to maximize participation for older adults. Across all sections, the central message is that a dementia diagnosis does not equate to incapacity. Early diagnosis safeguards legal autonomy, enabling individuals to articulate values and preferences while decisional abilities remain intact.

12. Between Autonomy And Safety: Involuntary Psychiatric Hospitalization In North Macedonia

Author: Ana Filipce (University Clinic of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Involuntary psychiatric hospitalization can protect patients and others from serious harm, but it also limits personal freedom. A proper decision should be based on national law, European human-rights standards, clinical judgment, and ethical principles. Legal permission alone is not enough.

Forensic relevance: The presentation reviews the legal framework for involuntary hospitalization in North Macedonia, together with the Winterwerp criteria under Article 5 of the European Convention on Human Rights and the five conditions described in Council of Europe Recommendation.

Assessment and management: Involuntary admission may be justified when a mental disorder is present, there is a serious risk of harm, no safe less restrictive option is available, and the patient refuses voluntary care. Legal safeguards should continue after admission, including information, legal representation, hearing, review, complaint, and regular reassessment. Involuntary admission does not automatically mean consent to treatment.

Conclusion: Following the law is necessary, but it is not enough. The decision must also be necessary, proportionate, and open to review. The CPT report following its 2023 visit to North Macedonia also highlighted gaps between formal safeguards and their implementation in practice. Possible improvements include a standard clinical–legal checklist, faster communication between hospitals and courts, regular audit, and better community-based alternatives.

Keywords: involuntary hospitalization, psychiatric law, human rights, risk assessment, North Macedonia.

13. Forensic Interview for Victims of Child Trafficking

Author: Liljana Ignjatova (Ss.Cyril and Methodius University in Skopje – Department of Psychiatry and Medical Psychology, PHI Psychiatric Hospital-Skopje, North Macedonia)

Interviewing child victims and witnesses of violence, exploitation, and trafficking requires a structured, child-centred, and non-suggestive approach. This paper presents findings on the use of different question types in interviews conducted with and without the National Institute of Child Health and Human Development (NICHD) Protocol, as well as differences in interviewing practices before and after forensic interviewer training. Particular attention was given to open-ended, directive/focused, closed, option-posing, and suggestive questions and their potential influence on the quality of children's statements. The findings highlight the importance of open-ended questions and free recall, while closed and suggestive questions may increase misunderstanding, guessing, and interviewer influence. Effective interviewing begins with appropriate preparation, informing the child, establishing rapport, and explaining interview rules, followed by practice in episodic recall and gradual transition to substantive issues. Interviewers should adapt language and communication to the child's age, developmental and communicative abilities, while considering the effects of trauma, fear, and previous experiences of violence. Appropriate protocol implementation and interviewer training can contribute to higher-quality and more reliable statements, reduce secondary victimization, strengthen child protection, and support more effective investigative and judicial procedures, and promote evidence-based, child-sensitive interviewing standards and professional practice across multidisciplinary child protection and justice systems.

Keywords: forensic interview, child victims, NICHD Protocol, question types, interviewer training, suggestibility, child testimony.

14. Insight, Violence And Psychosis: Treatment Options In Forensic Psychiatric Settings

Author: Zoja Babinkostova (University Clinic of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Insight is one of the many factors having an impact on a patient's tendency towards violent behavior. Researchers have traditionally divided insight into two concepts: clinical insight, defined as the awareness of a mental illness requiring treatment, and cognitive insight, defined as the ability to re-evaluate thoughts and beliefs and to resist self-certainty.

Violence is more prevalent in patients with psychotic disorders compared to the general population. Impaired insight is suggested as a risk factor for violence in psychosis.

Violence reduction is among the goals of treatment in general and emergency psychiatric care settings as well as in forensic psychiatry, where both pharmacological and nonpharmacological strategies are used. Adequate violence risk assessment is of high clinical importance. Examination of insight by validated instruments comprising different components may add useful information to clinical violence risk assessment in psychosis patients.

Positive symptoms, non-adherence to medication treatments, substance and alcohol abuse, and a history of aggressive behavior and personality disorders are only a few that enhance the chance of a patient without insight into their illness acting in a violent manner. On the other hand, negative symptoms and depression have been found to provide a protective role against violent behavior.

Regarding pharmacological treatment in forensic psychiatry settings in patients with aggressive behavior and violence, the most rational treatment choice is targeting the receptors that mediate these clinical manifestations. Parenterally administered antipsychotics represent the most frequently used drugs for the chemical restraint of aggressive patients. Additionally LAIs have been shown to reduce the frequency and severity of violence in patients with psychosis.

Keywords: insight, violence, psychosis, antipsychotics, LAIs.

15. Forensic Psychiatry In Comparative Context: System Differences And Clinical Challenges

Authors: Goran Arbanas (University Psychiatric Hospital Vrapče), Nuša Šegrec (University Psychiatric Clinic Ljubljana), Polona Rus Prelog (University Psychiatric Clinic Ljubljana), Hans Idenfors (Regional Forensic Psychiatric Clinic)

Forensic psychiatry operates at the intersection of clinical psychiatry, law, public safety, and human rights. Although forensic psychiatric patients often present with comparable clinical and risk-related problems across countries, the legal and organizational frameworks within which they are assessed, treated, supervised, and discharged may differ substantially.

This symposium will examine forensic psychiatry through a shared clinical-forensic case, analysed from the perspective of three national systems: Croatian, Slovenian, and Swedish. Using the same case example, the speakers will present how each system would approach criminal responsibility, risk assessment, court-ordered treatment or security measures, inpatient and outpatient care, discharge planning, and long-term follow-up.

Particular attention will be given to differences in the legal status of forensic patients, the availability of specialized forensic psychiatric services, the role of courts and social welfare institutions, and the challenges that arise when clinical needs do not correspond to available systemic solutions. The symposium will also address issues of continuity of care, rehabilitation, public protection, and the rights of persons with mental disorders.

By comparing three systems through one concrete case, the symposium aims to demonstrate how systemic differences directly influence clinical decision-making, treatment duration, discharge possibilities, and the overall position of forensic psychiatric patients.

16. Issues in Domain of Forensic Expertise of Patients Suffering of Querulous Delusion

Author: Opanković A, Milovanović S, Dunjić Kostić B, Miljević Č (Clinic for Psychiatry, UKCS, Belgrade, Serbia, Institute for Mental Health, Belgrade, Serbia, Faculty of Medicine, University of Belgrade, Belgrade, Serbia)

Querulous delusion represents a special challenge in the field of psychiatric expertise. Preserved formal thinking and partial criticality often mask a psychotic basis. The basic problem is reflected in demarcating the pathological fixation of insane beliefs from real life conflicts and legitimate legal demands. Of particular importance is the distinction between vindictive insanity from overvalued ideas and paranoid personality traits, as well as the assessment of the influence of the insanity system on decision-making relevant to legal actions. In such cases, a multidisciplinary approach is needed, with a longitudinal observation of psychopathology and careful evaluation of medical documentation.

17. Aggression and Violence in Schizophrenia

Author: Cedo Miljevic (Faculty of Medicine, University of Belgrade, Serbia)

Schizophrenia is one of the most severe mental disorders. It affects approximately 1% of the global population. The clinical picture of schizophrenia differs significantly between individuals and can also change markedly in the same person over time. No single symptom is present in every patient, which complicates the diagnosis of schizophrenia.

The public perception of the dangerousness of psychiatric patients is insidious, and stigma associated with schizophrenia is one of the most significant obstacles today in mental health treatment. Inadequate treatment is one of the major predictors of aggression in patients. Research indicates that only 10% of persons with a diagnosis of schizophrenia commit violent crimes.

Several risk factors contribute to aggression in patients. History of childhood aversive experience is one of the major risk factors. Comorbidity with substance abuse is the most important clinical indicator of increased aggressive behaviors in patients with schizophrenia. Genetic studies have proposed that polymorphisms in the promoter region of the serotonin transporter gene and in the catechol-O-methyltransferase gene are related to aggression. Neuroimaging studies have suggested that fronto-limbic dysfunction may be related to aggression or violence. Importantly, by identifying specific risk factors, a more efficient treatment plan to prevent violent behavior in schizophrenia will be possible.

18. Child Abuse-Related Fatalities

Authors: Goran Pavlovski, Natasha Bitoljanu, Viktorija Belakaposka-Srpanova, Ljupcho Chakar, Ana Ivcheva, Rosica Chakar, Zlatko Jakjovski, Aleksandar Stankov (Institute of Forensic Medicine, Criminalistics and Medical Deontology, Medical Faculty, Ss. Cyril and Methodius University, Skopje, North Macedonia)

Introduction: Child abuse represents a serious medical, social, and legal problem, with potentially long-term consequences for the child's physical and psychological health, development, and quality of life. From a forensic medical perspective, the recognition and appropriate interpretation of the injuries associated with abuse pose a particular challenge, as clinical and post-mortem findings may be subtle, nonspecific, or influenced by the child's age.

Aim: This paper presents the forensic medical characteristics of cases of child abuse resulting in death, with particular emphasis on the type and distribution of injuries, the mechanisms of their occurrence, associated pathological findings, and criteria for distinguishing inflicted injuries from accidental ones. It also highlights the importance of forensic medical examination and autopsy in the detection and documentation of child abuse.

Materials and Methods: Two cases of homicide committed by parents are presented. In both, the deaths were initially reported as accidental falls. A forensic autopsy was performed in each case, and the findings were analyzed in correlation with the available medical history and investigative data.

Results: In both cases, the cause of death was traumatic shock resulting from multiple injuries distributed throughout the body.

Conclusion: Child abuse resulting in death is a complex forensic medical problem requiring a systematic approach and multidisciplinary cooperation. The presence of multiple injuries of different ages, atypical injury distribution, inconsistent explanation of the mechanism of injury, and severe craniocerebral or internal injuries should raise suspicion of abuse. Forensic autopsy plays a key role in establishing the cause and manner of death, reconstructing the mechanism of injury, and providing objective evidence relevant to judicial proceedings. Early recognition and appropriate documentation of signs of abuse are essential for preventing recurrent victimization and protecting children.

Keywords: child abuse, filicide, child fatality, forensic autopsy, traumatic shock.

19. Criminal Responsibility In Kleptomania May Be Diminished By Impaired Impulse Control

Author: Ömer Bazna (Gülhane Training and Research Hospital, Department of Psychiatry)

Background: Kleptomania is a rare impulse-control disorder characterized by recurrent failure to resist urges to steal items of little personal or monetary value.

Case description/Clinical context: We present a 43-year-old married woman with a history of postpartum depression and chronic psychosocial stressors, including marital conflict and emotional deprivation. Over the past two years, she reported recurrent impulsive theft of low-value cosmetic items from shopping centers. These behaviors were consistently preceded by mounting internal tension, followed by a sense of relief during the act and marked guilt afterward. There was no evidence of premeditation, financial motivation, or antisocial intent.

Forensic relevance: The patient was referred for forensic psychiatric evaluation following repeated shoplifting incidents. Despite intact awareness of the illegality of her actions, her capacity to inhibit impulses was significantly compromised. This dissociation between preserved insight and impaired behavioral control raises critical challenges in forensic assessments, particularly in differentiating impulsive acts from deliberate criminal intent within legal frameworks.

Assessment and management: Psychometric evaluation revealed severe depressive symptoms (BDI:32), moderate anxiety (BAI:24), high impulsivity (BIS-11:52), and elevated kleptomania severity (K-SAS:37), along with obsessive-compulsive features (Y-BOCS:22). Treatment with fluoxetine 60 mg/day and supportive psychotherapy targeting compulsive traits resulted in partial clinical improvement and reduced frequency of theft behaviors.

Conclusion: This case supports the view that criminal responsibility in kleptomania may be diminished due to impaired impulse-control. Integrating structured psychiatric assessment into legal decision-making is essential, and treatment-oriented approaches should be prioritized over purely punitive responses.

Keywords: kleptomania, criminal responsibility, forensic psychiatry.

20. Психолошки Аспекти На Сајбер-Криминалот И Предизвиците За Форензичката Практика

Автори: Лариса Стојковић (Универзитетска Клиника по Психијатрија), Андромахи Наумовска (Институт за ментално здравје за деца и млади Младост)

Со забрзаниот развој на дигиталните технологии, сајбер-криминалот претставува една од најбрзо растечките форми на криминално однесување во современото општество. Покрај техничките и правните аспекти, сè поголемо значење добиваат психолошките фактори што влијаат врз мотивацијата, однесувањето и профилот на сторителите на дигиталните кривични дела. Дигиталната форензичка психологија претставува нова интердисциплинарна област која ги интегрира сознанијата од форензичката психологија, криминологијата, сајбер-безбедноста и бихејвиоралните науки со цел подобро разбирање, проценка и превенција на сајбер - криминалот.

Трудот се базира на преглед и анализа на современа научна литература и релевантни меѓународни истражувања од областа на форензичката психологија и сајбер - криминологијата. Анализата укажува дека анонимноста на интернет, намалената перцепција на одговорност, специфичните личносни карактеристики и когнитивните искривувања можат да придонесат за појава на криминално однесување во дигиталната средина.

Дигиталната форензичка психологија има сè позначајна улога и примената на интердисциплинарни пристапи и современи технологии, вклучително и вештачката интелигенција, ќе придонесе за поефикасно откривање, проценка и превенција на дигиталниот криминал.

Клучни зборови: дигитална форензичка психологија, сајбер-криминал, психолошко профилирање, дигитално однесување.

21. Community-Based Forensic Mental Health Care in North Macedonia: Risk Assessment, Safety, and Deinstitutionalization

Authors: Valentina Talevska (Psychiatric hospital Demir Hisar), Angela Ristevski (Bezirkskrankenhaus Bayreuth, Bayreuth, Germany)

Background: Community-based forensic mental health care enables treatment outside secure inpatient settings while maintaining professional supervision, legal safeguards, and public safety. Safe implementation requires clear eligibility criteria, structured risk assessment, proportionate security measures, and formal communication with courts.

Objective: To propose a framework for North Macedonia addressing patient eligibility, risk assessment, transition from compulsory inpatient treatment to community care, court reporting, and safe deinstitutionalization.

Methods: A structured narrative review of peer-reviewed literature, systematic reviews, comparative studies, professional judgment manuals, and WHO and WHO/OHCHR guidance was conducted. It focused on community forensic services, compulsory treatment, conditional discharge, risk and protective factors, judicial cooperation, and human rights.

Results: Admission should be based on multidisciplinary assessment of clinical stability, dynamic risk factors, treatment adherence, substance use, housing, social support, legal status, and service capacity. HCR-20V3, START, and SAPROF may improve transparency but should not replace individualized professional judgment. Community restrictions should be proportionate, legally grounded, linked to identified risk, and regularly reviewed. Transition from inpatient care should include a crisis plan, suitable accommodation, judicial review when required, and intensive follow-up. Court reports should include clinical status, adherence, incidents, risk and protective factors, and recommendations.

Conclusion: North Macedonia needs a coordinated health and legal framework linking forensic inpatient units, community mental health centers, social services, and courts. Deinstitutionalization should create safe, monitored, rights-based community alternatives.

Keywords: forensic psychiatry, community mental health, risk assessment, security measures, deinstitutionalization.

22. Fitness To Stand Trial In Psychotic Disorders: Clinical-Forensic Criteria, Common Pitfalls And The Razum Framework

Author: Predrag Mišić (SBPB Gornja Toponica, Serbia)

Background: Fitness to stand trial is the current, functional ability to understand criminal proceedings and take a rational part in one's defence. A psychotic diagnosis alone does not prove unfitness, while intact orientation or learned legal facts do not prove effective participation.

Objective: To set out clinical-forensic criteria for assessing defendants with psychotic disorders, describe common errors, and propose a clear and legally defensible framework.

Methods: A critical narrative review of legal rules, expert guidance and relevant literature was combined with experience from institutional clinical and forensic practice.

Results: Assessment should examine understanding of charges, possible outcomes, courtroom roles and procedure; attention and memory for relevant facts; rational communication and cooperation with counsel; and legal choices not directly driven by delusions, hallucinations or severe thought disorder. Special focus is needed on paranoid ideas about the court or counsel, poor reality testing, disorganised thought, negative and cognitive symptoms, mood symptoms, substance use, possible malingering and change over time. The proposed RAZUM framework covers five domains: Referral question and legal issue; Actual psychopathology; Knowledge and understanding of proceedings; Understanding and appreciation of one's own legal situation; and Meaningful capacity for rational defence. RAZUM is an author-made guide for reasoning, not a test or validated scale.

Conclusion: Opinions should be individual, time-specific and based on function. A defensible opinion must link each relevant symptom to a trial-related ability, separate difficult cooperation from true incapacity, and state limits and conditions for reassessment.

Keywords: fitness to stand trial, psychotic disorders, forensic psychiatry, expert evaluation, functional assessment.

23. Folie À Machine: Delusions Co-Constructed With Conversational Artificial Intelligence And The Question Of Criminal Responsibility

Author: Predrag Mišić (SBPB Gornja Toponica, Serbia)

Background: Consider a constructed illustrative scenario, a composite drawn from published reports: over several months a patient with an emerging delusional system exchanges messages daily with a conversational AI system. The interlocutor affirms each successive elaboration, never once contradicts, and remains continuously available. When the patient is later referred for forensic evaluation, the transcript is the only real-time record of how the delusion formed. No such case has yet been reported in the Serbian forensic literature; internationally, the first accounts have begun to appear.

Aim: To examine whether interaction with a generative language model can occupy a role analogous to that of the inducing person in shared delusional disorder (F24), and what follows for assessment of diminished and abolished criminal responsibility.

Method: Structured review of published cases and commentaries (2023-2026), with normative analysis under the Criminal Code of Serbia.

Results: Unlike classical folie à deux, the machine interlocutor has no psychopathology of its own, does not share the delusion and cannot bear responsibility - yet it satisfies the criterion of sustained, uncritical affirmation within a close and isolating relationship. A provisional typology of affirmation mechanisms is proposed. Under Articles 23 and 24 of the Criminal Code such material does not alter the biological criterion, but materially changes assessment of the psychological criterion: it permits reconstruction of the delusional trajectory in real time, ordinarily unavailable to the expert witness.

Conclusion: Transcripts of interaction with AI systems are becoming an independent source of forensic evidence. Minimum standards for their collection, authentication and evaluation are proposed.

Keywords: folie à deux; artificial intelligence; criminal responsibility; forensic psychiatry; digital evidence.

24. Forensic Psychiatric Aspects of the Criminal Offense of Incitement to Suicide – Case Report

Authors: Olivera Subotin Popovska, Vilma Videnova, Saska Simonovska Petrovska (Psychiatric Hospital Skopje, N.Macedonia)

Introduction: Incitement to suicide is a complex forensic phenomenon requiring assessment of the mental state of the deceased, suicidality, and psychosocial stressors. Long-term exposure to domestic psychological and physical violence can lead to affective disturbances, helplessness, hopelessness, and increased suicide risk.

Case report: A woman was subjected to prolonged psychological and physical abuse by her husband, leading to deterioration of mental functioning and psychosocial adaptation. In this chronic traumatizing context, a suicidal crisis developed, culminating in a suicide in which her child also lost its life. The forensic-psychiatric evaluation reviewed the nature and duration of the violence, the victim's mental state, and her behavior before the event, focusing on the interaction between vulnerability and chronic psychotrauma.

Discussion: The case demonstrates the complexity of assessing suicidality in prolonged domestic violence. Chronic exposure eroded personal control, intensified helplessness and hopelessness, and narrowed perceived options for escape. During acute escalation of violence, cognitive-affective rigidity further limited risk assessment and evaluation of consequences. The analysis emphasizes reconstructing the victim's mental state over time and understanding the dynamic relationship between violence, suicidal crisis, and the final outcome.

Conclusion: A multidimensional forensic-psychiatric approach is essential in cases involving incitement to suicide. Prolonged domestic trauma may influence perception of danger, evaluation of alternatives, and decisionmaking during a crisis. In this case, available data align with intense emotional distress and cognitive-affective constriction immediately before the event. Longitudinal analysis of violence, psychological state, suicidality, and interpersonal dynamics enhances understanding of forensic factors preceding the suicidal outcome.

Keywords: suicide, domestic trauma; forensic factors.

25. Addiction Treatment And Mental Health In Custodial Settings

Authors: Andrej Kastelic (Medical and Project Consultation), Jorg Pont (Medical University Vienna), Heino Stoever (Institute for Addiction Research (ISFF), Frankfurt University of Applied Sciences, Frankfurt am Main, Germany, EUex)

Mental health disorders including substance use disorders are significantly more prevalent in prison populations than in the general population, with interventions like cognitive behavioural therapy (CBT), peer support programs and trauma-informed care showing considerable effectiveness in reducing symptoms. The research also highlights the critical role of social determinants and the importance of systemic reforms, such as reducing solitary confinement and integrating prison health care with broader public health systems, in improving inmate mental health and reducing recidivism.

People who use drugs in prisons are a vulnerable population even more than other prisoners. They are over represented in prisons and have complex health needs: infectious diseases and psychiatric comorbidities. There is high mortality rate in prison due to suicide and high mortality rate after prison release (overdose deaths).

Two important principles for health interventions in prison are equivalence of care to that provided in the community and continuity of care between the community, prison and other closed settings including police arrest on admission and after release.

Opioid agonist (substitution) treatment (AOT/OST) is protective against death in prison for opioid-dependent prisoners, important in reducing injecting risk behaviors, and prevents overdose death in the period directly following prison release. Together with psychosocial treatment reduces re-incarceration rates in drug-using offenders. Education and training interventions with naloxone provision help to decrease overdose-related deaths after release.

Drug court programs can help people in social reintegration. Quasi-coercive treatment can be as effective as voluntary treatment.

From: KASTELIC A, PONT J, STOEVER H. Clinical foundation for the use of opioid agonist treatment in prison. in: MAREMMANI, Icro (ed). Principles and practice of methadone treatment. Pietrasanta: AU-CNS Press, 2026.

26. Unusual Suicidal Deaths: A Forensic Case Series

Authors: Ljupcho Chakar, Goran Pavlovski, Natasha Bitoljanu, Viktorija Belakaposka Srpanova, Gligor Ristovski, Ana Ivcheva, Rosica Chakar, Zlatko Jakjovski, Aleksandar Stankov (Institute Of Forensic Medicine, Criminalistics And Medical Deontology, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, North Macedonia)

Introduction: Although many suicidal deaths involve relatively recognizable methods, some cases involve unusual or highly complex acts that may provide insight into the diversity of suicidal behavior among individuals. In forensic practice, such cases require careful multidisciplinary interpretation of the circumstances, method of suicide, and individual characteristics.

Aim: To present four unusual suicidal deaths recorded in the forensic practice of the Institute of Forensic Medicine in Skopje, to highlight the diversity of suicidal methods, mechanisms of death, and circumstances associated with these fatalities.

Methods: A retrospective case-series analysis was conducted using the Institute's electronic databases and archives. Four cases were selected based on the unusual nature of the suicidal method or mechanism of death. Available documentation was analyzed, including information on the circumstances of the event, scene findings, autopsy findings, histopathological and toxicological results, and relevant medical documentation. The cases originated from different years, with the earliest dating from 2017.

Results: The four cases included fatal methanol poisoning following ingestion of windshield-washer fluid; traumatic decapitation following a fall from height onto a metal fence; suicidal electrocution involving electrical cables attached to both upper limbs, with a potato found in the mouth; and a fatal penetrating craniocerebral injury caused by driving a nail into the head using a hammer. The psychiatric and behavioral context, when available, provided important additional information for understanding the circumstances surrounding the suicidal acts.

Conclusion: Although the manner of death was established as suicide in all cases, interpretation of the findings required integration of information regarding the circumstances of the event, available health records, autopsy findings, and ancillary investigations. Awareness of atypical suicidal methods may contribute to a more comprehensive understanding of suicidal behavior and support multidisciplinary collaboration among the professionals involved in death investigation.

Keywords: suicide; forensic medicine; electrocution; methanol; toxicology; autopsy.

27. Forensic Patient Who Perpetrated Homicide Or Attempted Homicide Treated At The Demir Hisar Psychiatric Hospital During The Period 2025-2025

Authors: Pavlina Ilieska-Kotevska (Psychiatric Hospital Demir Hisar, Demir Hisar), Kosta Sotiroski (Ekonomski fakultet Prilep)

Background: The PHI Psychiatric hospital Demir Hisar provides treatment within its forensic psychiatry department for patients subjected to a compulsory psychiatric treatment and custody measure due to a court decision. Some of these patients are perpetrators of criminal offences involving homicide or attempted homicide and are treated within the forensic psychiatry department.

Aim: The aim of this study was to analyze the demographic and clinical characteristics of forensic patients who perpetrated homicide or attempted homicide and were treated at the Psychiatric hospital Demir Hisar during the period 2015-2025.

Methods: A total of 34 patients (29 men and 5 women) who were treated at the Psychiatric Hospital were included in the study. The patients were analyzed according to sex, age, ethnicity, place of residence, education. marital status and diagnosis. Additional variables related to the criminal offence were analyzed including the manner in which homicide was committed, the type of weapon, the relationship with the victim.

Results: The obtained data indicate a higher proportion of homicide -related criminal offences in certain cities and an increase in the number of such cases was observed during the most recent time period analyzed.

Conclusion> The findings indicate the need for targeted preventive interventions and improved early identification and management of high -risk forensic psychiatric patients.

Keywords: Forensic patients, homicide, attempted homicide, forensic psychiatry, psychiatric treatment, prevention.

28. Forensic Psychiatry - Themes And Dilemmas From Clinician` Perspective

Authors: Pavlina Ilieska-Kotevska, Aleksandar Kotevski, Stevce Cvetanovski (Psychiatric Hospital Demir Hisar, Demir Hisar)

Introduction: Forensic psychiatry lies at the intersection of psychiatry, medicine, and the legal system. In North Macedonia, forensic patients requiring compulsory psychiatric treatment are treated in specialized psychiatric institutions. The complexity of this population, associated safety risks, and legal requirements present major challenges for clinicians.

Aim: To present key challenges in forensic psychiatric practice, focusing on patient safety, cooperation with judicial institutions, interinstitutional coordination, and opportunities for resocialization and rehabilitation.

Materials and Methods: A descriptive overview of challenges identified through clinical practice in forensic psychiatric departments, focusing on organizational, safety, legal, and therapeutic aspects of patient care.

Results: Six major areas were identified: inadequate infrastructure and safety conditions; difficulties in communication and cooperation with courts; insufficient coordination among specialized psychiatric institutions; the need to modernize the legal framework and develop community-based forensic services; challenges in treating patients with dual diagnoses; and the need for continuous treatment, risk assessment and management, resocialization, and rehabilitation.

Conclusion: Forensic psychiatry faces complex clinical, organizational, safety, and legal challenges that require a systemic and multidisciplinary approach. Improving infrastructure and safety standards, strengthening cooperation with the judicial system, enhancing interinstitutional coordination, modernizing legislation, and developing rehabilitation programs are essential for effective care. A coordinated approach is needed to ensure humane, safe, effective, and legally compliant treatment of forensic patients.

Keywords: forensic psychiatry; forensic patients; risk assessment; dual diagnosis; resocialization; rehabilitation.

29. Abrupt Clonazepam Discontinuation In Custody: Severe Benzodiazepine Withdrawal Delirium With Violent Behaviour

Authors: Vesna Suklovska, Aleksandar Kotevski, Stevce Cvetanovski, Pavlina Ilieska Kotevska (Psychiatric Hospital Demir Hisar, Demir Hisar)

Background: Abrupt discontinuation of long-term, high-dose benzodiazepines may cause severe withdrawal with delirium, perceptual disturbances, behavioural dyscontrol and violence, creating major clinical and forensic challenges.

Case description/Clinical context: A male detainee with long-term clonazepam use of up to approximately 20 mg/day had the medication abruptly discontinued after detention for theft. Within several days, he developed agitation, insomnia, disorientation, disorganized behaviour, self-directed and other-directed aggression, inappropriate urination and defecation, and behaviour suggestive of hallucinations. Shortly after admission to a forensic psychiatric unit, he became extremely aggressive, apparently responding to perceptual disturbances, and damaged a radiator.

Forensic relevance: The disturbance occurred in custody and resulted in severe aggression and property damage, emphasizing the need to distinguish withdrawal states from primary psychotic disorders in forensic settings.

Assessment and management: Toxicological analysis was negative for other psychoactive substances. The temporal relationship to clonazepam cessation and fluctuating presentation supported benzodiazepine withdrawal delirium (ICD-10 F13.4). Diazepam 30 mg/day was initiated with symptomatic antipsychotic treatment. Progressive improvement followed, with resolution of severe behavioural disturbance and disorientation and restoration of meaningful verbal contact.

Conclusion: Abrupt cessation of high-dose benzodiazepines in custody may cause severe neuropsychiatric symptoms and increase the risk of violence. Medication reconciliation, early recognition of benzodiazepine dependence and medically supervised withdrawal are essential to prevent delirium and avoidable forensic complications.

Keywords: clonazepam, benzodiazepine withdrawal, delirium, forensic psychiatry, aggression.

30. Reactive-Impulsive Violence In The Context Of Chronic Alcohol Dependence And Executive Dysfunction

Author: Ljubica Popova (Specialist in Medical Psychology and Forensic Expert - Ministry of Defence Skopje)

Background: Chronic alcohol dependence, combined with psychosocial stressors and cognitive changes, may significantly impair emotional and behavioural control.

Case Description/Clinical Context: The case concerns a 56-year-old man with long-term alcohol dependence, epilepsy, and multiple life stressors. On the day of the critical incident, he consumed alcohol at several locations. The immediate trigger was a minor family conflict. Approximately two hours after the incident, his blood alcohol concentration was 2.43‰. The examinee reported partial amnesia for the critical period.

Forensic Relevance: No indicators of prior planning or weapon preparation were identified. The behavioural pattern was more consistent with reactive-impulsive behaviour.

Assessment and Management: Neuropsychological assessment revealed preserved nonverbal reasoning and visuoconstructive abilities, but reduced visual memory and marked executive dysfunction. The personality profile indicated emotional instability, anxiety, depression, and severe alcohol dependence.

Conclusion: Chronic alcohol dependence, acute intoxication, and impaired executive functioning likely contributed to significantly reduced cognitive and emotional control at the critical moment.

Keywords: alcohol dependence; executive functions; impulsivity; forensic psychology; reactive violence.

31. Current Position Of Forensic Psychiatrists In Serbia's Judiciary

Authors: Zoran Ciric (court expert for forensic psychiatry - Faculty of Law, University Nis), Ivana Tešanović (psychiatrist)

Legal amendments in Serbia brought changes in the role of forensic psychiatrist. Now the parties in the proceedings are enabled to hire a forensic psychiatrist, who should provide them with indirect professional assistance, by actively participating in proceedings. This ensures the control of expert psychiatrists, critical approach to the expertise and presence of them in the procedures of expertise.

In criminal law this expert is called an "expert advisor" who has the right to attend the expert examination, to suggest certain procedures and, after finishing expertise, to comment on the findings and opinion of the expert, to question them at the main trial and to give explanations about expertise. Also, the advisor has the right to be questioned about the circumstances of his opinion.

In civil law, it is enabled to hire a forensic psychiatrist to express objections to the expert's findings, to make his own findings, to examine the expert psychiatrist at the trial and to give clarifications about issues of the expert opinion. If their findings are different, the court is obliged to order a new expert opinion.

In practice, unfortunately, these experts are not well accepted by the prosecutors and judges. They consider that the expert adviser, in any case, will speak in a sense that suits one who hired him. Lawyers, it seems, did not properly understand the basic intention of the law writer, to ensure greater objectivity and a critical attitude towards experts by hiring experts of the same profile, so expert advisors give help to all participants of procedure.

We hope that practice will enable the proper application of existing solutions, that it will influence the exercise of control over experts and prevent the potential abuse that expert consultants may commit. For all the above reasons, it is necessary to organize meetings of lawyers in the judiciary and court psychiatrists, with the aim to reduce disagreements, misunderstandings and the exclusivity of one or the other.

32. Clinical - Forensic Characteristics In Patients With Behavioral Disorder As A Result Of Alcohol And Other Psychoactive Substance Use

Authors: Ana Stankovska-Gjorgieva (PHI Psychiatric Hospital Demir Hisar), Liljana Ignatova (Ss.Cyril and Methodius University in Skopje – Department of Psychiatry and Medical Psychology, PHI Psychiatric Hospital-Skopje, North Macedonia)

Background: Behavioral disorders resulting from the use of alcohol and other psychoactive substances (PAS) represent a significant public health issue and are frequently associated with forensic proceedings and compulsory treatment.

Objective: To present the sociodemographic, clinical, and forensic characteristics of patients with behavioral disorders related to psychoactive substance use.

Materials and methods: A retrospective, descriptive study was conducted on the medical records of 37 patients treated at the PHI Psychiatric Hospital Demir Hisar, with data processed up to May 2026. Age, gender, education, marital and employment status, diagnosis according to ICD-10, type of court-ordered measure, and the court of origin of the measure were analyzed. Descriptive statistics were used.

Results: All 37 patients were male, and the most prominent age group was 40–49 years (54.1%). Eighteen patients (48.6%) had a single diagnosis—the most common being disorders related to alcohol use (F10), registered in 11 patients, while 19 (51.4%) had dual disorders-the most frequently occurring other mental disorders alongside substance use disorders were F60–F69 disorders. The most common court-ordered measure was mandatory psychiatric treatment and confinement in a health institution in 17 patients (45.9%), and most of the measures originated from the District Court Bitola (56.7%).

Conclusion: The examined population is dominated by middle-aged men, with a significant prevalence of disorders related to alcohol use and a high proportion of dual disorders. The results emphasize the need for an integrated approach to treatment, rehabilitation and community reintegration after the completion of the measure, aiming to improve the quality of life in this population.

Keywords: PAS (psychoactive substances), court-ordered measure,dual disorders.

33. Alcohol-Induced Blackout and Criminal Responsibility: A Forensic Psychiatric Case of Acute Aggression

Author: Paula Pazos Suárez (Hospital La Paz)

Background: Alcohol-induced blackouts are characterized by impaired memory formation during acute intoxication and are frequently invoked in forensic settings after alcohol-related violence. However, blackout does not necessarily imply loss of consciousness or global cognitive incapacity. Distinguishing amnesia from impairment of understanding or behavioural control is therefore relevant to criminal responsibility.

Case Description: A 20-year-old man was brought to the emergency department by police after a physical altercation in a public setting. He presented with marked alcohol intoxication, disinhibition, verbal and physical aggression, and poor cooperation. After recovery, he reported complete amnesia for the period surrounding the incident. Police and witness accounts indicated that he had remained awake, interacted purposefully with others and responded to the immediate situation. No psychotic, manic or neurological syndrome was identified.

Forensic Relevance: The case raised the question of whether an alcohol-induced blackout could support impaired criminal responsibility. Under Spanish criminal law, complete exemption related to intoxication requires that intoxication prevent the individual from understanding the unlawfulness of the act or acting accordingly. Therefore, retrospective amnesia alone cannot establish such incapacity.

Conclusion: Alcohol-induced blackout should not automatically be equated with loss of criminal responsibility. Forensic assessment should distinguish memory impairment from impairment of consciousness, cognition and behavioural control, integrating contemporaneous clinical observations, toxicology and collateral information.

Keywords: alcohol blackout; criminal responsibility; forensic psychiatry; acute intoxication; aggression

34. Repeated Traumatic Experiences as a Trigger for a Homicide in the Heat of the Moment

Authors: Vesna Jovanovska (General Hopistal Kumanovo, North Macedonia)

Background: Prolonged exposure to domestic violence may result in significant psychological vulnerability and may affect a person's psychological functioning during episodes of acute and life-threatening danger. This case presents the psychological assessment of a 45-year-old woman exposed to prolonged physical and psychological domestic violence.

Case Description/Clinical Context: For years, the woman had been exposed to repeated physical assaults, threats, psychological abuse, and sexual coercion by her husband, with additional violence and threats from her father-in-law. She had repeatedly sought assistance from the police and social welfare services and had previously been registered as a victim of domestic violence. On the day of the incident, she was attacked and, fearing for her life, killed both men with a knife.

Forensic Relevance: The psychological assessment indicated that the woman had been exposed to cumulative and escalating trauma. During the critical incident, heightened fear, anger, arousal, and perceived threat to her life culminated in intense affective tension and a brief narrowing of consciousness, which reduced her capacity for logical reasoning and for clearly understanding the significance of her actions.

Assessment and Management: The psychological evaluation included intelligence testing, the MMPI, PIE, assessment of psychological defense mechanisms, stress and aggression testing, the NEO-PI-R, and the Machover test. The findings indicated average intellectual abilities, an absence of deeper progressive psychopathology, neurotic and subdepressive symptoms, anxiety, helplessness, and pronounced vulnerability.

Conclusion: The case illustrates the psychological consequences of chronic domestic violence and the potential role of accumulated trauma and acute physiological affect in behavior occurring under conditions of immediate life-threatening danger.

35. Neurobehavioral Heterogeneity Of Aggressive Behavior In Children: Candidate Electrophysiological Markers And Implications For Forensic Psychiatric Assessment

Authors: Silvana Markovska-Simoska (Macedonian Academy of Sciences and Arts), Bojana Boshkovska (Neurodevelopmental Center “In My world”, Skopje, North Macedonia)

Background: Childhood aggression is a significant clinical concern, yet the pathways leading to aggressive behavior are far from uniform. While aggressive acts are readily observable, the neurocognitive and emotional systems underlying them often differ markedly across individuals. This heterogeneity has important implications for forensic assessment—identical aggressive behaviors may reflect fundamentally different regulatory vulnerabilities, risk trajectories, and intervention targets.

Case description/Clinical context: Five children with significant aggressive and oppositional presentations underwent integrated clinical assessment including developmental history, behavioral observation, standardized behavioral measures, and neurocognitive evaluation. Quantitative EEG (QEEG) and event-related potentials (ERP) were obtained to examine patterns of arousal, attention, inhibition, and conflict monitoring.

Forensic relevance: Across cases, aggressive behavior consistently reflected distinct regulatory pathways rather than a single pathophysiological profile. Some children showed primarily impulsive, disinhibited presentations; others displayed heightened threat sensitivity and emotional reactivity; and several exhibited cognitive rigidity or poor environmental adaptation. This distinction is essential for forensic formulation: aggressive behavior itself provides limited insight into mechanism, intent, or underlying risk.

Assessment and management: Resting QEEG abnormalities—including increased slow-frequency activity and altered theta/beta relationships—appeared to index general vulnerability to dysregulation rather than aggression per se. In contrast, task-based ERP measures (N2 and No-Go P3) showed more direct relevance to inhibitory and conflict-monitoring deficits, aligning with known mechanisms of reactive aggression. A multimodal approach combining spectral and event-related electrophysiology may better capture individual variation in regulatory vulnerability.

Conclusion: Aggressive behavior in children represents a common final pathway of distinct neurobehavioral trajectories. Forensic assessment must account for this heterogeneity. Systematic investigation of multimodal electrophysiological profiles in larger samples may help differentiate regulatory pathways, refine mechanism-based risk formulation, and guide targeted intervention.

Keywords: aggression, heterogeneity, ERP, QEEG, forensic assessment.

36. Forensic Psychological Evaluation in the Context of a Proposal for Outpatient Treatment

Authors: Ena Canevska (EAPTI Gestalt Institut - Skopje), Andromahi Naumovska (Institute for Mental Health of Children and Youth Mladost - Skopje)

Forensic psychological assessment in the context of involuntary outpatient treatment requires integration of current psychological functioning, previous psychopathology, and factors associated with future decompensation. Cases such as the present one are particularly challenging when no active psychotic symptoms are identified despite a history of a psychotic episode with violent behavior.

This case presents a 49 years old woman with higher education and stable occupational functioning, involved in prolonged marital conflict with elements of domestic violence. Anamnestic data indicate chronic stress, a single psychotic episode associated with an incident involving physical injury and subsequent psychiatric hospitalizations, followed by prolonged legal and interpersonal conflicts after divorce. The psychological evaluation was conducted within proceedings concerning proposed outpatient treatment, focusing on current psychological functioning, reality testing, and emotional regulation.

Findings indicate above average intellectual abilities, preserved reality testing, and no evidence of current psychotic symptomatology or psycho-organic impairment. Thought processes are organized and coherent. Elements of emotional tension, depressed mood, mistrust, and a tendency to perceive the environment as potentially threatening are noted. Difficulties in affect regulation and control of aggressive impulses emerge under increased stress, with tendencies toward withdrawal and potential impulsive reactions.

The findings indicate a significant impact of chronic interpersonal stress on emotional functioning without impairment of cognitive capacities or reality testing, highlighting the importance of considering contextual stressors in forensic psychological evaluations concerning outpatient treatment.

Keywords: forensic psychology, domestic violence, outpatient treatment, emotional regulation.

37. Neuropsychological Assessment in a Forensic Context

Authors: Katerina Gruevska (University Clinic of Neurology – Skopje), Andromahi Naumovska (Institute for Mental Health of Children and Youth Mladost - Skopje)

Neuropsychological assessment has become an important source of evidence in legal settings where questions about cognitive functioning, psychological status, and the consequences of neurological injury arise. Its role is not limited to identifying cognitive difficulties; rather, it provides a structured framework for examining how an individual's functioning may relate to the legal question under consideration. This is particularly relevant when there are concerns about the accuracy of symptom reporting, possible exaggeration of difficulties, or the influence of psychological factors on observed performance.

A neuropsychological assessment draws on information from several complementary sources - standardized neuropsychological measures, clinical interviews, behavioral observations, third-party reports, questionnaires, and medical or legal records. Integrating these sources allows the examiner to evaluate patterns of functioning and determine whether the findings are consistent across different forms of evidence.

In civil and administrative matters, neuropsychological findings may be particularly relevant when an individual reports cognitive or functional difficulties following a traumatic brain injury. Assessment findings can help clarify the extent to which an injury is associated with changes in cognitive functioning and everyday abilities. They may consequently inform decisions concerning functional limitations, disability, work capacity or compensation.

In criminal proceedings, the findings may provide relevant information on a defendant's ability to understand and participate in legal proceedings, factors relevant to criminal responsibility or mitigation, and potential risk.

Overall, forensic neuropsychological assessment provides an evidence-based framework for connecting patterns of brain functioning and behavior with questions that arise in civil, criminal, and administrative contexts.

Keywords: neuropsychological assessment, legal settings, cognition.

38. Akutni Psihoticni Poremecaj Sa Nasilnim Ispadom Kod Pacijenta Sa Parkinsonovom Bolescu

Authors: Maja Petrovic (Pirot General Hospital, Serbia), Ljiljana Simic (Clinic for Mental Disorders Dr Laza Lazarevic, Serbia)

Background: Psychotic symptoms are common in Parkinson's disease (PD), with psychosis incidence ~35% (Alsharaeh et al., 2024) and point prevalence of hallucinations 28% (Ravandi et al., 2023). Visual hallucinations affect 25-30% (Fénelon et al., 2000). Psychotic decompensation may be triggered by disease, therapy, or stressors, leading to violence (Laurell et al., 2023; Dutta, 2023).

Case description: A 65-year-old male with 10-year PD, hypertension, atrial fibrillation. Seven months pre-admission, visual perceptions. One month before, son died. Returning from the cemetery, he took a hoe and axe, attacking property, speaking incoherently. Escorted by police, agitated, inappropriate affect, lacking insight. During week one, fluctuating consciousness, disorientation, sweating, subfebrility – delirium. After 3 weeks, recovered, psychosis resolved, but remained unaware of son's death.

Forensic relevance: Attending psychiatrist initiated forensic evaluation via public prosecutor, recommending compulsory psychiatric treatment. Key dilemma: did organic psychotic disorder/delirium exclude capacity to understand actions and control behavior? The patient was in acute psychotic decompensation with hallucinations, disorganized thinking, lacking insight - unable to understand actions.

Conclusion: This case highlights integrative assessment of violent incidents in PD patients and clinician's role in initiating forensic evaluation. Timely recognition of organic causes is crucial for clinical outcomes and forensic decisions. A humanistic perspective is essential - recognizing the patient as a person who, due to neurodegenerative disease and loss, could not control his actions, requiring treatment rather than punishment. Clinical risk assessment remains important but requires structured instruments.

Keywords: forensic psychiatry, Parkinson's disease, organic psychotic disorder, delirium, violence, criminal responsibility.

39. Violence Risk Assessment After Sustained Remission in a Patient With Psychotic Disorder: A Case Report

Authors: Ljiljana Simic, Maja Tomić, Marija Ilic (Clinic for Mental Disorders Dr Laza Lazarevic, Serbia), Maja Petrovic (Pirot General Hospital, Serbia)

Background: Violence risk assessment in forensic psychiatry requires integration of historical risk factors with current clinical and protective factors. Previous severe violence may continue to influence perceptions of risk despite sustained remission.

Case description/Clinical context: A 37-year-old man with psychotic disorder (ICD-10: F29, F22) experienced a psychotic relapse approximately ten years after an initial cannabis-associated episode. During the relapse, he fatally stabbed his grandfather. The court imposed compulsory hospitalization in a high-security forensic unit. Following several years of treatment, his condition stabilized, restrictions were reduced, and the measure was terminated.

Forensic relevance: Four months after termination, his history of lethal violence remained relevant in a family-law context, where concerns about risk were raised. The case illustrates the challenge of distinguishing historical violence from current risk.

Assessment and management: Evaluation focused on mental state, insight, treatment adherence, psychosocial functioning, family support, and dynamic and protective factors. The patient was stable, cooperative, and engaged in treatment. He had completed university, married, became a father, and was employed as an IT consultant. Stable family support, employment, adherence, and sustained remission were identified as protective factors. Risk was assessed through clinical evaluation and collateral information. Continued outpatient care and monitoring were recommended.

Conclusion: This case highlights the importance of individualized violence risk assessment. Although previous lethal violence remains a significant historical risk factor, sustained remission, treatment adherence, social support, employment, and community functioning may represent important protective factors.

Keywords: Forensic psychiatry, violence risk assessment, psychotic disorder, community reintegration, protective factors.

40. Beyond Fair Play: Forensic Psychiatric Perspectives In Competitive Sport

Authors: Aleksandar Pavlović (Serbian Institute of Sport and Sports medicine), Ljiljana Simic (Clinic for Mental Disorders “Dr Laza Lazarević”, Belgrade, Serbia), Maja Tomić (Clinic for Mental Disorders “Dr Laza Lazarević”, Belgrade, Serbia)

Background: Competitive sport involves intense physical and psychological demands and may expose athletes to situations in which aggression, emotional dysregulation, substance use or impaired judgement become clinically and legally relevant. The interface between sports medicine, sports psychiatry and forensic psychiatry remains insufficiently explored.

Clinical context: Athletes presenting with marked aggression, behavioural dyscontrol,doping related psychiatric symptoms may represent a complex clinical challenge. Competitive pressure, sport-specific norms and expectations of toughness can obscure underlying psychopathology. Clinical evaluation should therefore consider psychiatric history, substance use, impulse control, previous aggressive behaviour, psychosocial stressors and the circumstances surrounding the behavioural episode.

Forensic relevance: emerges when aggressive or disinhibited behaviour results in injury, violence, legal proceedings or questions concerning responsibility. The essential forensic question is the relationship between mental state, behaviour and the specific event.

Assessment and management: A multidisciplinary approach involving sports physicians, psychiatrist and forensic psychiatrists is recommended. Assessment should include psychopathology, substance use, violence and suicide risk, protective factors, contextual triggers and functional capacity. Management should prioritize safety, appropriate psychiatric treatment, substance-use intervention, risk reduction and coordinated follow-up.

Conclusion: The intersection of competitive sport and forensic psychiatry has important clinical and medicolegal implications. Forensic psychiatric principles can help distinguish normal competitive aggression from clinically significant behaviour requiring specialized or medicolegal evaluation.

Keywords: forensic psychiatry, sports psychiatry, aggression, competitive sport, risk assessment.

41. Credibility Assessment of a Child with Mild Intellectual Disability in Suspected Sexual Abuse: A Forensic Psychiatric Case Report

Authors: Biljana Arizankoska Eftimova (Center for Rehabilitation of Verbal Communication Pathologies - Skopje), Silvana Filipova (Center for Rehabilitation of Verbal Communication Pathologies - Skopje, Faculty of Pedagogy, University of Tetovo, Tetovo, Republic of North Macedonia), Biljana Maricik Jakimovska (Center for Rehabilitation of Verbal Communication Pathologies-Skopje)

Background: Assessing testimony of children with intellectual disabilities is challenging in forensic psychiatry. Cognitive and language limitations may affect recall and narrative precision but should not automatically indicate unreliability.

Case description: An eight-year-old girl with mild intellectual disability underwent forensic psychiatric and psychological evaluation following alleged sexual abuse by a family member. She showed concrete thinking, limited vocabulary, articulation and grammatical difficulties, reduced comprehension of complex questions, and difficulties with temporal orientation and sequencing. Immediate memory was relatively preserved, enabling recall of personally experienced events within her cognitive limitations.

Assessment: The evaluation included psychiatric and psychological examination, cognitive and adaptive assessment, and a developmentally adapted, non-leading forensic interview. When verbal expression was insufficient, the child supplemented her account through non-verbal demonstration. Despite difficulties with chronology and peripheral details, the core elements of the alleged event remained consistent. No indicators of confabulation or spontaneous fabrication were observed.

Conclusion: Mild intellectual disability does not preclude a child from providing forensically meaningful information. Developmental difficulties in chronology, language, and peripheral details should not alone be interpreted as evidence of unreliability. Individualized assessment and adapted forensic interviewing are essential for balanced credibility assessment.

Keywords: child sexual abuse; intellectual disability; child testimony; credibility assessment; forensic psychiatry.

42. Challenges In Assessing Parental Capacities: Between Objective Assessment And The Best Interests Of The Child

Author: Ljubinka Damjanovikj (ZD Skopje)

Introduction: The evaluation of parental capacities during high-conflict custody, divorce, and child placement disputes represents a highly sensitive and challenging domain within modern forensic psychiatry and psychology. The core difficulty stems from the reality that parenting capacity is never a permanent trait, but a highly fluid, responsive dynamic deeply affected by ongoing situational stress.

Objective: This study systematically examines the primary methodological, practical, and clinical obstacles that forensic multidisciplinary teams encounter. It aims to bridge the gap between objective clinical evaluations and the application of the ultimate legal standard, which is the absolute best interest of the child.

Methods and Results: Through case analysis and comparative literature review, the study maps critical systemic challenges. The findings highlight widespread parental manipulation strategies, including intentional impression management ('goodness simulation') and the severe psychological dynamics of parental alienation. Furthermore, evaluation efforts are regularly hindered by external institutional pressures for immediate black-and-white outcomes and a distinct, critical shortage of standardized forensic diagnostic tools that are fully validated for the local sociocultural context.

Conclusion: High-quality, reliable expert testimony must abandon general clinical assumptions and rely strictly on multi-setting behavioral observations and rigorous forensic protocols. Eliminating evaluator bias is essential to effectively extract and protect the authentic, uninfluenced voice and emotional well-being of the vulnerable child within the legal system.

Keywords: parental capacity, forensic assessment, best interests of the child, parental alienation, family law.

43. Beyond the Psychosis Paradigm: Antisocial Personality Disorder and Criminal Responsibility in Double Grandparent Homicide

Authors: Viktorija Vujovikj, Stefanija Mitrovska (University Clinic of Psychiatry, Faculty of Medicine, Ss Cyril and Methodius, Skopje, North Macedonia)

Background: Homicide of a close ascendant relative is a rare and clinically complex form of intrafamilial violence. Although the parricide literature has emphasized severe mental disorders, particularly psychosis, personality and substance-use disorders are also represented. This case illustrates the distinction between psychiatric diagnosis and its functional impact on criminal responsibility.

Case presentation: A 28-year-old man killed both grandparents, who had been his primary caregivers since childhood, by repeatedly striking them with a blunt object in their home. He was intermittently employed and financially dependent on the victims. He reported habitual alcohol use and smoking but denied other substance use. Forensic evaluation involved repeated assessments by two psychiatrists and a clinical psychologist. His presentation was quiet, brief and affectively flat, with limited elaboration on the offence. No psychotic symptoms or acute substance-induced state were identified, and reality testing was intact. Blood alcohol concentration was 0.00‰, excluding acute alcohol intoxication. Antisocial personality disorder was diagnosed.

Discussion: The forensic formulation considered enduring personality pathology, interpersonal dependency, behavioral decision-making and offence circumstances. Reported habitual alcohol use was distinguished from intoxication at the material time. The expert opinion identified no psychiatric grounds for exculpation or diminished responsibility and no mitigating circumstances attributable to mental disorder. The verdict was intentional homicide, with life imprisonment.

Conclusion: Antisocial personality disorder may help explain interpersonal functioning and violent behavior, but does not itself imply diminished responsibility. Forensic conclusions should rest on the functional relationship between psychopathology and the relevant capacities at the time of the offence, rather than on diagnosis alone.

Keywords: Antisocial personality disorder; parricide; criminal responsibility; forensic psychiatric assessment; homicide.

44. Beyond the Violent Act: What Lies Behind Severe Impulsive Violence in Adolescents? Developmental, Psychiatric, Personality, Family and Social Factors

Authors: Biljana Arizankoska Eftimova (PHI Center for Rehabilitation of Pathology of Verbal Communication - Skopje, North Macedonia, Faculty of Medical Sciences, Goce Delcev University, Stip, North Macedonia), Slavica Arsova Hadji Angjelkovska (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia), Marija Darkovska Serafimovska (Faculty of Medical Sciences, Goce Delcev University, Stip, North Macedonia), Gordana Nikolova (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia)

Background: Severe impulsive violence in adolescence represents a complex clinical, forensic and public health challenge. Focusing solely on the violent act may overlook the developmental and psychosocial mechanisms underlying such behavior.

Aim: To examine the interaction of developmental, psychiatric, personality, family and social factors associated with severe impulsive violence in adolescents.

Methods: A focused narrative review of relevant literature on adolescent violence, developmental psychopathology, personality functioning and juvenile delinquency was conducted.

Results: Adolescent violence is best understood as a multidetermined outcome rather than the consequence of a single disorder. Developmental immaturity, impulsivity and difficulties in emotional regulation may interact with externalizing psychopathology, substance use and exposure to trauma. Impairments in personality functioning, including difficulties in identity, self-regulation, empathy and interpersonal functioning, may provide an additional dimension for understanding individual vulnerability, but should not be equated with violence or dangerousness. Family conflict, inconsistent parenting, poor supervision, delinquent peers, social normalization of aggression and access to weapons may further increase risk. The accumulation and interaction of risk factors appear more informative than any single factor.

Conclusion: Severe violent behavior in adolescence is not a diagnosis but a possible outcome of interacting developmental and psychosocial vulnerabilities. Assessment should move beyond categorical diagnosis toward an individualized formulation integrating developmental maturity, psychopathology, personality functioning, family and social context. Such an approach may support more targeted prevention and intervention.

Keywords: adolescent violence; personality functioning; impulsivity; juvenile delinquency; forensic psychiatry.

45. Who Let the Dogs Out? Forensic Psychiatric Aspects of Dog Bites in North Macedonia

Authors: Bojan Nikolovski, Aleksandar Kjosev (PHI General Hospital Strumica), Petar Sotiroski, Antoni Novotni, Elena Dimovska, Ljubisha Novotni (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia)

Background: Attacks by free-roaming dogs are an increasingly common reason for forensic medical expert assessments in civil claims for non-pecuniary damage in North Macedonia. According to records from the Ministry of Internal Affairs, 229 dog bites were recorded in the City of Skopje only in the first four months of 2025, while the numbers for the year 2024 reach 880 reports of dog bites. Media reports citing data from the City of Skopje indicate that substantial amounts of compensation were paid to citizens for non-pecuniary damage related to dog attacks in 2025.

Observations: Claimants, through their legal representatives, increasingly request forensic assessment of physical pain, fear and diminished capacity for daily life activities, and in some cases disfigurement. Most cases involve minor bodily injuries (bite wounds and abrasions). Serious bodily injuries also occur, such as long-bone fractures of the limbs when dogs attack people riding motorcycles or bicycles. A growing number of victims seek psychiatric care for symptoms that follow a dog attack, with clinical presentations potentially including acute stress symptoms, anxiety, avoidance behaviour, specific phobia and, in some cases, post-traumatic stress disorder (PTSD). The psychological consequences may be particularly relevant in children.

Conclusion: Preventive measures aimed at reducing encounters between citizens and free-roaming dogs could potentially reduce the associated physical and psychological burden, as well as the resulting costs to public institutions.

Keywords: dog bites; free-roaming dogs; non-pecuniary damage; forensic medicine.

46. Alcohol Involvement In Road Traffic Accident Cases: Postmortem Findings And Retrograde Bac Assessment

Authors: Rosica Chakar, Goran Pavlovski, Ljupcho Chakar, Natasha Bitoljanu, Viktorija Belakaposka Srpanova, Ana Ivcheva, Zlatko Jakjovski, Aleksandar Stankov (Institute Of Forensic Medicine, Criminology And Medical Deontology, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Alcohol use and dependence represent a significant public health and socioeconomic problem; consequently, blood alcohol concentration (BAC) is frequently assessed during forensic medical examinations and investigations. Alcohol intoxication impairs sensory, cognitive, psychomotor, and behavioral functions - effects that are particularly relevant in road traffic incidents. Retrograde extrapolation of BAC tempore criminis is a common request from forensic experts in both criminal and civil proceedings.

Objective: To assess alcohol involvement among deceased road traffic participants and to review the forensic application and limitations of retrograde BAC extrapolation in medico-legal cases.

Methods: A retrospective descriptive study was conducted using the Institute's internal electronic database for the years 2021–2025, searched using relevant keywords. A separate retrospective review was performed on medico-legal court cases from the Institute's archive in which retrograde BAC extrapolation had been requested.

Results: The analysis of autopsy cases included 175 deceased road traffic participants. Blood ethanol was detected in 25 cases (14.3%), including 5 pedestrians (8.2%), 13 drivers (19.4%), and 7 motorcyclists (22.5%); aged 17-79 years, all male except for one female driver. No alcohol - positive cases were identified among cyclists. Postmortem blood ethanol concentration ranged from 0.15–1.51‰ in pedestrians, 0.41–3.63‰ in drivers, and 0.28–2.67‰ in motorcyclists. A review of medico-legal cases identified several factors affecting retrospective BAC assessment, including calculations based on Dräger breath-test results, prolonged intervals, and marked discrepancies between blood and urine ethanol concentrations.

Conclusion: Postmortem ethanol analysis provides important information on alcohol involvement in fatal road traffic incidents but requires careful interpretation because postmortem processes may affect ethanol concentrations. Retrograde BAC extrapolation is a complementary forensic tool whose reliability depends on the phase of alcohol kinetics, timing of sampling, available evidence, and assumptions used in the calculation. Appropriate biological sampling and validated analytical methods are essential for reliable forensic alcohol assessment.

Keywords: blood alcohol concentration (BAC), ethanol intoxication, retrograde calculation, forensic toxicology, road traffic.

47. European Psychiatric Association Code Of Ethics: Ethical Guide For Psychiatrists In Europe

Authors: Goran Račetović, Meryam Schouler-Ocak (Health Centre Prijedor, Community Mental Health Center, Prijedor, Republic of Srpska, Bosnia and Herzegovina; Committee on Ethics, European Psychiatric Association)

Background: European Psychiatric Association (EPA) developed Code of Ethics as a universal guideline for psychiatrists in Europe with the aim to provide protection of human rights of people with mental health disorders as one of important factors for good outcomes and better involvement of patients in their communities. After revision in 2024, EPA Code of Ethics is harmonized with the Code of Ethics by the World Psychiatric Association (WPA).

As one of activities of the EPA Committee on Ethics, dissemination of Code of Ethics is a way to present ethical principles to European psychiatrists during major national events and realized in collaboration with EPA Council of the National Psychiatric Associations (EPA NPAs). Also, translation of EPA Code of Ethics to European languages is a way for psychiatric associations across Europe to harmonize their codes or adopt EPA Code of Ethics as their national ethical principles and support protection and best possible recovery of their patients.

Keywords: European Psychiatric Association, Ethics, Dissemination.

48. Testamentary And Financial Abuse Of Older Adults With Dementia In Long-Term Care: Lessons From Three Cases

Authors: Gordana Ristevska Dimitrovska (Higher Medical School - University St. Kliment Ohridski Bitola, North Macedonia), Jane Ristevski (PZU Psihomedika, PUSZ Dom Sofija – Bitola, North Macedonia)

Background: Population ageing and emigration are increasing the demand for long-term residential care in North Macedonia. Older adults with cognitive impairment are especially vulnerable to financial exploitation, including undue influence over wills and property transfers. Such abuse may remain undetected because of family involvement and fragmented institutional oversight.

Case description: Dom Sofija is a 70-bed long-term care facility in Bitola, where residents with dementia comprise approximately 30% of the capacity. During four years of operation, the multidisciplinary team encountered three cases of suspected financial and testamentary abuse. Relatives attempted to exploit residents' cognitive decline, dependency and social vulnerability to obtain substantial assets or property. All three residents had lived abroad for a considerable period and had adult children residing abroad.

Forensic relevance: The cases raised questions about testamentary capacity, vulnerability to undue influence, safeguarding responsibilities and the evidentiary value of clinical records. Forensic psychiatric expertise contributed to two cases, including one retrospective post-mortem assessment, enabling the suspected abuse to be challenged or prevented. In the third case, the resident was transferred to another facility without adequate communication with social services, limiting opportunities for intervention.

Assessment and management: Cognitive and functional assessment, documentation of changes in expressed wishes, recognition of family pressure, and timely cooperation among psychiatrists, nurses, social workers and legal authorities were essential. The cases also exposed gaps in continuity of care and interinstitutional communication.

Conclusion: Long-term care professionals are well positioned to recognize financial and testamentary abuse. Clear safeguarding protocols and coordinated medico-legal responses are needed to protect residents with impaired decision-making capacity.

Keywords: dementia, financial abuse, testamentary capacity, undue influence, long-term care

49. Aggressive Behaviour In Paranoia: A Case Report

Authors: Imran Memedi (University of Tetovo), Kristina Vasilevska (JZU Psychiatric Hospital Skopje), Selma Arifi (University of Tetovo)

Introduction: Although mental illnesses account for only a small proportion of aggressive behavior in the general population, the manifestation of aggressive behavior in patients with paranoid psychopathology may be particularly concerning. Aggressive behavior in paranoia is often directed towards family members, particularly during the early stages of illness and when the condition remains untreated.

Methods: We report the case of a 32-year-old man whose symptoms had emerged approximately 10 months prior to presentation. Clinical data, forensic assessments, mental health institutional records, and community-based rehabilitation reports were reviewed to construct his clinical and forensic trajectory.

Results: The patient exhibited verbal and physical aggression towards his wife, driven by suspicions that she was unfaithful and involved in relationships with other men. In addition to paranoid psychopathology, the consumption of alcohol and psychoactive substances was associated with impaired control of his aggressive and impulsive behavior. The patient was diagnosed with persistent delusional disorder (F22), and mental and behavioral disorders due to cocaine use, harmful use (F14.1), ICD-10. The clinical picture indicated the need for regular psychiatric treatment. According to the forensic psychiatric assessment, tempore crimini, the patient's behavior was driven by psychopathological experiences, and he was considered unable to control his actions or understand the significance of his conduct and the resulting violence.

Conclusion: Paranoid psychopathology may increase the risk of aggression toward intimate partners and family members, particularly when compounded by substance use and antisocial behavior. Early identification of risk factors, appropriate psychiatric treatment, and effective substance-use management may help reduce recurrent violence and improve the safety of patients and their families.

Keywords: Aggressive behavior, paranoia, family members.

50. When Delusions Have Consequences: Epistemic Uncertainty and Third-Party Risk in Clinical Psychiatry

Authors: Vaska Damjanovska Mitova, Slavica Arsova, Simona Kochoska Krstevska, Adriana B. Toskic, Stafanija Mitrovska, Marko Naumovski, Stojan Bajraktarov (University Clinic of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Delusional disorder can coexist with preserved cognition, social functioning, and apparently intact everyday decision-making. Ethical complexity increases when delusion-driven behavior may expose others to harm while information remains partly verifiable.

Case presentation: We describe a middle-aged man with preserved thought organization and intellectual functioning, but longstanding systematized delusions involving grandiose, persecutory, and health-related beliefs, absent insight, and impaired judgment within the delusional domain. For years, he used self-prepared metal-containing substances, including silver-based preparations, believing they had therapeutic properties. Marked grey-blue skin discoloration was evident. He reported similar use among close contacts and treatment of others. Collateral information partly supported this exposure, but broader claims regarding third-party treatment could not be independently verified. Antipsychotic treatment was initiated, but adherence was poor.

Discussion: The treating psychiatrist faced information suggesting possible harm to others, although its reliability and extent remained uncertain because part of the account was embedded within a delusional system. This creates epistemic uncertainty: neither dismissing the information as delusional nor accepting it as fact is defensible without corroboration. Confidentiality, autonomy, decision-making capacity, non-maleficence, and third-party protection therefore come into tension.

Conclusion: Delusional content is not evidence by itself, but uncertainty does not imply absence of risk. The case illustrates the difficulty of determining when partially corroborated concerns become sufficiently credible to justify proportionate action beyond the therapeutic relationship.

Keywords: delusional disorder; epistemic uncertainty; third-party risk; confidentiality; decision-making capacity; forensic psychiatry; clinical ethics.

51. Ефикасноста на Алтернативна Мерка Условна Осуда со Заштитен Надзор во Третманот кај Хероински Зависници на Супституционен Третман

Автори: Анета Спасовска Трајановска (ПБ Скопје), Зора Митиќ (ПЗУ Зора Митиќ), Љ. Спировска

Вовед: Хероинската зависност е доста рапространета болест во светски размери и претставува значаен медицински, но и социјален проблем. Познато е дека употребата на хероинот покрај бројни здравствени проблеми како од соматска така и од психичка природа доведува и до појава на криминални дејанија. Во ефикасноста на третманот во оваа болест, покрај супституцината терапија, односно фармакотерапија, од голема важност е и психосоцијалниот третман. Во успешно спроведување на оваа активност, свој придонес има и примената на алтернативна мерка-условна осуда со заштитен надзор, со која лекарот кој го води пацинетот (хероински зависник) е задолжен во период од шест месеци да дава писмен извештај за неговата здравствена состојба до подрачната пробациска канцеларија, која е посебна единица во рамките на Министреството за правда (Управа за извршување на санкции).

Цел: да се согледа ефикасноста на казнената-алтернативна мерка во стабилизацијата на хероински зависници со супституционен третман.

Материјал и методи: Проспективната студија беше спроведена во период од една години во Дневната болница за зависности при ЈЗУ Психијатриска болница Скопје. Беа евалуирани 15 пациенти од различен пол, со средна возраст од 27±6 години: пред почеток на спроведување на казнено-алтернативната мерка по три месеци и по шест месеци од нејзиното спроведување. 10 пациенти беа на третман со метадонска терапија во дневна доза Sol metadon 9-12 ml, а 5 пациенти со Ling Buprenorphine 24-28 mg дневна доза, без друга дополнителна терапија. Пациентите беа следени со стнадизиран прашлник специјално дизајниран за студијата во кој беа нотирани: возраст, пол, брачна состојба, статус на вработеност, години на образование, фекфенција на криминални активности. Употребата на ПАС беше одредувана со помош на телесен преглед (свежи убоди од и.в. апликација на ПАС), како и скрининг на урина за квалитативно присуство на ПАС. Добиените податоци беа одредувани со помош на Дескриптивна метатода и Студентов т-тест. За статистичка значајност беше земена вредноста $p < 0,05$.

Резултатите во нашата студија укажаа дека по шест месечено спроведување на казнено алтернативна мерка имаше некое мало подобрување кај испитаничата група во однос на нивната стабилизација, но без статистичка значајност $p=0,45$ додека по една година од спропведување на казнено алтернативната мерка се постигна одговор со статистичка значајност $p=0,03$.

Заклучок: со примената на казнено алтернативната мерака по една година кај хероински зависници на третман со супституциона терапија се постигна релативно добра стабилизација.

52. Epilepsy Patients Rights

Authors: Natalija Dolnenec-Baneva, Dragana Petrovska-Cvetkovska (University Clinic For Neurology, Skopje, N.Macedonia), Evgenija Baneva (University Clinic For Radiotherapy And Oncology, Skopje, N.Macedonia)

Background: Over the past few decades, great strides have been made in diagnosing and treating epilepsy, a neurological disorder characterized by recurring seizures. Despite these advances, epilepsy continues to evoke negative stereotypes, fear and discrimination. Persons diagnosed as having epilepsy may suddenly find that they are treated differently and sometimes unfairly: Qualified workers may be denied job opportunities due to stereotyped views about risks for seizures in the workplace; colleges may refuse to make reasonable academic adjustments, such as scheduling changes, needed to ensure successful participation; people experiencing a common type of seizure may be erroneously viewed as aggressive or intoxicated, and be unfairly arrested and/or injured by first responders; schools and day care programs may refuse to administer emergency seizure medication. This issue describes some of the legal issues for a person who has been diagnosed as having epilepsy or for someone`s family member with epilepsy. When somebody faces any of these problems, the issue helps to know epilepsy patients legal rights.

Conclusion: There are many differences between European and domestic epilepsy patients rights programs. Much needs to be done by preparing epilepsy programs based on discrimination which provide legal guidance to individuals with epilepsy and their families, along with referrals to a nationwide network of lawyers who can provide advice and representation (including a free initial consultation).

Keywords: epilepsy, patient rights, legal guidance.

53. Cognitive Behavioral Therapy (CBT) in Forensic Psychiatry: Challenges in Clinical Practice

Authors: Dijana Miloseva, Zoja Babinkostova (University Clinic of Psychiatry, Medical faculty, Department of Psychiatry and Medical Psychology, Ss. Cyril and Methodius, Skopje, North Macedonia)

Background: Although Cognitive Behavioral Therapy (CBT) is regarded as the gold standard in the realm of clinical psychiatry, its implementation within forensic psychiatry presents particular challenges, especially concerning the specifics of the patients and the institutional environment itself.

Aim: The purpose of this paper is to explore the viability of incorporating CBT within the realms of forensic psychiatry and psychotherapy, as well as to deliberate on the necessary adaptations required to accommodate the unique clinical and institutional characteristics of forensic populations.

Methods: The paper is grounded in a comprehensive literary review and synthesis of the principles and clinical applications of CBT in forensic treatment, with particular emphasis on the latest methodologies and evidence-based findings derived from globally published research and clinical experiences. To this end, we utilized the following databases: PubMed, Cochrane Library, and Scopus.

Results: The results reveal that the principles of CBT can be integrated into forensic psychiatric treatment; however, the implementation necessitates greater flexibility than is typically required in conventional outpatient settings. CBT may prove particularly beneficial when combined with broader psychiatric and psychosocial interventions, especially when the treatment goals are directly linked to the risk factors associated with criminal behavior and violence. Effective interventions should be structured, individualized, and specific in nature. Institutional factors, staff collaboration, and sustainable continuity of care represent significant determinants of successful implementation.

Conclusion: CBT is potentially applicable in forensic psychiatry. Successful implementation necessitates adaptation to the forensic context, individualized treatment planning and integration with risk management and psychiatric care.

Keywords: cognitive behavioral therapy; forensic psychiatry; forensic psychotherapy.

54. Предизвици со кои се Соочуваат Вештите Лица од Областа на Медицината при Вештачење во Граѓанската и Кривичната Постапка

Автор: Зора Митиќ (ПЗУ Зора Митиќ, Скопје)

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Најважни законски обврски на медицинскиот вештак: Член 2 од Законот за вештачење го дефинира вештакот како физичко лице запишано во регистарот на вештаци за соодветната област. Вештачењето е стручен наод и мислење засновано на научни и стручни методи, квалификации и знаење.

Заклучок: Врз основа на сето изнесено се наметнува потреба од подобра комуникација и соработка на вештите лице меѓусебно и со органите во комората за изнајдување и создавање на одредени протоколарни решенија.

55. Effects of Pharmacological ADHD Treatment on Criminal Recidivism and Rehabilitation in Inmates With ADHD

Author: Sasa Jevtovic (University Hospital Centre Zagreb, Croatia)

Attention-deficit/hyperactivity disorder (ADHD) is a neurodevelopmental disorder with childhood onset that is characterized by patterns of inattention and/or hyperactivity–impulsivity that interfere with daily life functioning. These patterns often persist into adulthood and may be associated with increased risks of educational and occupational failure, antisocial behavior, and criminality.

There is a high prevalence of attention-deficit/hyperactivity disorder (ADHD) in prison populations compared to the general population, and ADHD has also been shown to be associated with criminality and antisocial behavior.

Adults with ADHD often suffer from other mental health disorders, including anxiety disorders, mood disorders, substance use disorder (SUD), and various behavior disorders. A meta-analysis has shown that inmates with ADHD, in comparison with inmates without ADHD, exhibit a higher incidence of behavioral problems and substance use but also antisocial personality disorder. Inmates with ADHD are reported to be involved more frequently in severe aggressive incidents during imprisonment and they also tend to exhibit an earlier onset of crime, more reported incidents of violence, as well as relapsing into crime more often and faster after release compared to other inmates.

Pharmacological treatment of ADHD may constitute an important component of evidence-informed correctional mental health care; however, its specific contribution to reducing criminal recidivism and improving long-term rehabilitation remains to be established. Evidence suggests that these treatments can reduce ADHD symptoms and enhance rehabilitation outcomes, which may, in turn, lower the rate of reoffending.

Future research should prioritize adequately powered prospective studies, standardized recidivism measures, and longitudinal assessments of treatment continuity and post-release outcomes. Clarifying the relationship between symptom control, functional improvement, and reoffending is essential for determining the role of pharmacotherapy within comprehensive rehabilitation strategies for incarcerated individuals with ADHD.

56. Femicide: A Medico-Legal Case Series on Forensic Findings and Investigative Relevance

Authors: Viktorija Belakaposka Srpanova, Natasha Bitoljanu, Goran Pavlovski, Ana Ivcheva, Ljupcho Chakar, Rosica Chakar, Zlatko Jakjovski, Aleksandar Stankov (Institute for forensic medicine, criminalistics, and medical deontology, Medical Faculty, Skopje, University of "Ss Cyril and Methodius", North Macedonia)

Background: Femicide is the intentional killing of women or girls in a gender-related context and represents an extreme form of gender-based violence. It frequently occurs within intimate or family relationships and may follow previous violence, coercive control, or other abuse. From a forensic perspective, recognition of relevant circumstances and injury patterns is essential for accurate classification, reconstruction, and judicial investigation. International frameworks developed by UN Women and UNODC emphasize systematic documentation of victim, perpetrator, relationship, circumstances, and forensic findings.

Case description: We present medico-legal cases of suspected femicide examined at the Institute of Forensic Medicine, Criminalistics, and Medical Deontology. The adult female victims differed in victim–perpetrator relationship, circumstances, mechanisms of fatal injury, and forensic findings.

Forensic relevance: Comparative evaluation demonstrated the importance of integrating autopsy findings with crime-scene investigation, and toxicological examination, and information concerning interpersonal circumstances. Documentation of injury distribution, previous violence, defensive injuries, and circumstances indicating a gender-related context may assist in distinguishing femicide from other forms of female homicide and reconstructing events.

Conclusion: These cases highlight the central role of forensic medicine in documenting and investigating suspected femicide. A multidisciplinary and standardized approach, integrating medico-legal, laboratory, investigative, and judicial information, may improve recognition of gender-related killings and support accurate classification and judicial investigation.

Keywords: femicide, forensic medicine, female homicide, forensic pathology, gender-based violence.

57. Deconstructing Insight And Judgement In Forensic Psychiatry: Conceptual Vagueness

Authors: Jane Ivanoski, Julia Gjinoska, Egzon Kurtishi (University Clinic of Psychiatry, Skopje, N. Macedonia), Kocho Bojadziev (Psychiatric Hospital Demir Hisar, N. Macedonia), Marko Trajkovski (PHI Polyclinic Zelezara, Skopje, N. Macedonia), Slave Toshevski (PHI Psychiatric hospital Skopje, Skopje, N. Macedonia), Kadri Hadzihamza (Faculty of Medicine, Ss. Cyril and Methodius, University in Skopje, Skopje N. Macedonia)

Background: Clinical psychiatry routinely pairs "insight" and "judgement" in mental status examinations, yet multi-language literature and clinical perspectives expose profound conceptual ambiguity. Fundamental questions regarding their core definitions, mutual independence, and cross-linguistic alignment remain poorly delineated. Unraveling these clinical dilemmas is vital before applying them in forensic evaluations.

Objective: To analyze the core definitions, functional separability, and forensic implications of insight and judgement across domestic and international literature, complemented by practitioner perspectives, establishing a clearer framework for medicolegal practice.

Methods: A qualitative, multi-source study combined literature synthesis with practitioner inquiry. Core psychiatric, psychological, and forensic literature was reviewed across Macedonian, Balkan and Anglo-American traditions. This was complemented by semi-structured consultations with mental health professionals, including senior psychiatrists, psychiatry residents, and clinical psychologists.

Results: Thematic synthesis indicates that routine clinical descriptors (e.g., "intact," "impaired") oversimplify insight and judgement. Findings demonstrate they are distinct yet interconnected constructs: insight provides the reflective foundation for reality testing, whereas judgement translates self-awareness into situational reasoning and behavioral choices. In forensic contexts, both map fundamentally onto the cognitive dimension of sanity and accountability, showing that granular assessment of each is essential to evaluate how pathology impairs criminal responsibility.

Conclusion: Forensic psychiatric evaluations must establish distinct operational boundaries for insight and judgement rather than treating them as a single entity. Disentangling their dynamic relationship elevates expert testimony, ensuring rigorous assessments of legal capacity.

Keywords: insight, judgement, accountability.

58. From Diagnosis to Function: Mental Disorder, Reasoning and Authorship

Author: Marija Kostadinovska (Zan Mitrev Clinic, Skopje, Norht Mcedonia)

Background: Mental disorders may affect cognitive functions, reasoning, and decision-making, but do not necessarily imply their permanent or complete impairment. Authorship provides a specific context in which creation of a work, recognition of oneself as its author, understanding of copyright, and practical exercise of these rights can be distinguished.

Aim: To examine reasoning and selected psychological functions in persons with mental disorders and their influence on understanding authorship and choices related to exercising these rights.

Methods: A descriptive-comparative study included 94 adults: 51 with mental disorders—18 psychotic, 16 affective, and 17 substance-use disorders—and 43 without mental disorders, with artistic, scientific, or innovative authorship. A substantial part of the clinical sample was assessed in psychiatric institutions treating severe and chronic conditions. A purpose-designed questionnaire and selected VITIM subtests were used.

Results: The mean score for knowledge of authorship and copyright was approximately 80% in both groups, with no statistically significant difference. Participants recognized authors' moral and economic rights. Subgroup differences emerged in the practical use of the work, evaluation of alternatives, and concrete decision-making. The most pronounced functional difficulties were observed in the psychotic subgroup.

Conclusion: The findings support a shift from a diagnostic to a functional approach. Creating a work, recognizing oneself as its author, understanding related rights, evaluating alternatives, making decisions, and exercising rights in practice represent distinct and potentially dissociable functions. Forensic assessment should therefore determine what a person can understand, where functional difficulties exist, and what support is needed, rather than allowing diagnosis alone to justify restrictions on autonomy and rights.

59. Evaluating Parental Capacity and Attachment Trauma in High-Conflict Custody Disputes: A Case Report

Authors: Marko Naumovski, Nensi Manusheva, Zoja Babinkostova, Magda Manojlovska (University Clinic of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Interparental conflict and parental alienation represent critical psychosocial risk factors that may contribute to emotional dysregulation and pediatric somatization. Prolonged exposure to post-divorce interparental tension and devaluation may hinder socio-emotional development and contribute to severe loyalty conflicts in children.

Case Description/Clinical Context: We present a case of a 7-year-old child exposed to chronic, high-conflict custody litigation. Following the divorce, the mother demonstrated low frustration tolerance, oppositional behavior, and difficulties separating the former partner conflict from her parental role, accompanied by behaviors contributing to a negative paternal image. The child developed recurrent psychosomatic manifestations and acute stress symptoms immediately before scheduled visitations, expressing marked resistance despite exhibiting intact adaptive functioning in the school environment. The somatic distress was resolved shortly after the contacts were canceled. Forensic evaluation of the mother revealed intact overall mental status and adequate care for the child's physical and educational needs. However, it highlighted a reduced capacity to separate partner conflict from parental responsibilities. The father's evaluation confirmed an intact psychiatric status, preserved integrative capabilities, and adequate parenting capacity.

Forensic Relevance: This case underscores the diagnostic challenge of differentiating authentic pediatric phobia or anxiety from symptoms occurring within a context of parental alienation, where the child's somatic and emotional responses may contribute to the obstruction of parental contact.

Assessment and Management: Multidisciplinary evaluation indicated an urgent need for cognitive-behavioral interventions and parental psychoeducation for the mother, alongside supportive counseling for the father. The objective is to reduce the child's anxiety and resistance, restore the parent-child relationship, and gradually implement supervised contacts where clinically indicated.

Conclusion: Despite ongoing alienation dynamics, maintaining custody with the mother is currently recommended, as she represents the primary attachment figure, thereby avoiding the potential trauma associated with an abrupt transition. However, immediate therapeutic intervention is necessary to address maternal attitudes and behaviors and preserve the child's relationship with the father as an essential component of healthy emotional and identity development. Specifically, the evaluation recommends maintaining custody with the mother while immediately initiating regular, professionally supervised, and structured contact with the father, with the frequency and level of supervision adjusted according to the child's psychological response and ongoing professional assessment.

Keywords: Parental alienation, Child custody evaluation, Loyalty conflict, Attachment theory.

60. When Suicide Defies Obvious Interpretation: Two Cases of Complex Suicides in Forensic Psychiatry

Author: Djilali Merzoug Mohammed (Faculty of medicine)

Introduction: Complex suicides, characterized by the combination of multiple lethal methods, represent a major diagnostic challenge in forensic psychiatry. They raise difficulties in interpreting intentionality and the dynamics of events, particularly in elderly individuals.

Methods: We report two cases of complex suicides occurring in elderly individuals. The analysis is based on forensic autopsy findings, macroscopic examinations, and contextual elements collected during the investigation.

Results: In both cases, multiple suicidal mechanisms were identified. The victims presented with injuries to the neck and thorax, associated with self-inflicted wounds to the forearms. Ingestion of a caustic substance, specifically an insecticide, was also demonstrated, leading to severe gastrointestinal perforations. All anatomo-pathological and contextual findings supported the conclusion of complex suicides without third-party involvement.

Discussion: These observations illustrate the complexity of multi-method suicides, often associated with strong determination or fear of failure. Forensic analysis must be rigorous to exclude any criminal hypothesis.

Conclusion: Complex suicides require a multidisciplinary approach integrating clinical, forensic, and contextual data for reliable interpretation.

Keywords: complex suicide, psychiatric disorders, associated organic lesions.

61. Forensic Analysis of Homicides Committed by Individuals with Schizophrenia

Authors: Ana Ivcheva, Goran Pavlovski, Viktorija Belakaposka Srpanova, Gligor Ristovski, Natasha Bitoljanu, Ljupcho Chakar, Rosica Chakar, Zlatko Jakjovski, Aleksandar Stankov (Institute for forensic medicine, criminalistics, and medical deontology, Medical Faculty, Skopje, University of "Ss Cyril and Methodius", North Macedonia)

Background: Homicides committed by individuals with schizophrenia represent complex cases of relevance to both forensic medicine and psychiatry. Although schizophrenia itself does not imply violent behavior, the presence of active psychotic symptoms may be relevant to the forensic assessment of a particular event.

Case description/clinical content: A series of homicide cases committed by individuals diagnosed with schizophrenia is presented, in which the victims were most commonly members of the immediate family with whom the perpetrators lived. Complete forensic autopsies were performed on the victims. The number, localization, dimensions, morphology, direction, and depth of the injuries were analyzed, as well as the involvement of vital organs and blood vessels. In the majority of cases, stab wounds caused by a sharp-edged and pointed instrument, most commonly a knife, were identified. Information regarding the perpetrators' psychiatric history, treatment, and behavior immediately preceding the critical events was obtained from the investigating authorities.

Forensic relevance: Morphological analysis of the injuries enables identification of the causative instrument and reconstruction of the mechanism of injury. Correlation of autopsy findings with information from the scene contributes to determining the cause and mechanism of death.

Assessment and management: The assessment included systematic documentation of injuries, forensic autopsy examination, and toxicological analysis. An integrated forensic medical and forensic psychiatric approach enables comprehensive reconstruction and interpretation of these complex cases.

Keywords: Schizophrenia; Homicide; Forensic psychiatry; Forensic medicine.

62. Кога Мерката Безбедност Задолжително Лекување, Не е Доволна

Автори: Јасмина Степаноска, Мирјана Савеска (ГОБ „8-ми Септември“ - Скопје)

Вовед: Форензичко-психијатриски приказ на пациент со шизофренија кој извршил убиство за време на извршување на мерка безбедност.

Опис на случај: Обвинетиот боледува од шизофренија, 12 години издржува казна мерка безбедност во затворена Психијатриска установа. Првото дело за кое е обвинет е „Убиство“ на мајка на бизарен начин. За време на извршување на првото кривичното дело бил „Непресметлив“ и е предложена и прифатена Мерка безбедност задолжително лекување во Психијатриска Болница. За време на хоспитализацијата со агресивно, деструктивно однесување, често со флоридна психотична симтоматологија, со одбивање на терапија злоупотреба на алкохол и ПАС и чести бегства од Болницата.

Форензичко значење: За време на извршување на кривичното дело „Убиство“ имало редукција на психопатолошката содржина, големо влијание имало дезинхибиторното дејство на алкохолот кој го конзумирал претходно, така што тој дејствувал импулсивно, агресивно и деструктивно, кога не можел во доволна мера да го сфати значењето и последиците од делото и не можел да управува со своите постапки, или од правен аспект бил со битно намалена пресметливост.

Проценка и третман: Раниот почеток, тежината на болеста, непримањето на терапија во болнички услови злоупотребата на алкохол и ПАС, обвинетиот го прават опасен за себе и околината, поради што е предложено: издржување на казната во Пенални услови со препорака; при евентуално реактуелизирање на болеста и влошување на психичката состојба да се хоспитализира на затворено одделение и надзор од службени лица. Кај обвинеиот постои опасност да го повтори и по трет пат делото.

Потребна е индивидуализирана и настански специфична форензичко психијатриска проценка насочена кон сторителот на кривичното дело со шизофренија. Не дијагнозата туку функционалното влијание на психотичното нарушување во моментот на дело тој ја определува пресметливоста. Потребна е проценка на ризикот од насилство кај пациент со шизофренија кога е под „Мерка на безбедност задолжително лекување во Психијатриска Болница“ и неопходно е во судските одделенија во Психијатриските Болници да има службени лица.

Клучни зборови: шизофренија, пресметливост, убиство.

63. Assessing Criminal Responsibility in Schizophrenia: A Case Report

Authors: Nevena Nikolova, Zoja Babinkostova (University Clinic of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Introduction: In Macedonian criminal law, imputability relies on two criteria: biological and psychological. While a psychotic disorder satisfies the biological component, the psychological criterion concerns the capacity to understand the act and to control one's conduct at the time of the offence (tempore criminis). In judicial practice, a chronic schizophrenia diagnosis is often mistakenly conflated with continuous legal incapacity.

Methods: A three-member board from the Forensic Medicine Board at the Faculty of Medicine in Skopje performed a court-ordered super-expertise for a defendant in his thirties charged with aggravated robbery. The assessment aimed to resolve two conflicting prior evaluations: one finding no mental disorder with preserved imputability, and a second diagnosing undifferentiated schizophrenia and concluding non-imputability. A third prior evaluation had exclusively addressed the defendant's fitness to stand trial. The board conducted a direct clinical examination and systematically reviewed the court file, previous forensic opinions, and seven years of longitudinal psychiatric records.

Results: The first hospitalisation was documented as acute schizophrenia-like psychotic disorder (F23.2). The diagnosis was later changed to undifferentiated schizophrenia (F20.3), with negative symptoms predominating. Outpatient reports from the period of the offence describe an unchanged state, with no exacerbation or reactualisation of psychotic symptoms. Both patient and collateral histories indicated normal functioning on the day of the event. The defense rested on simple denial of the act, not on psychotic content. At the time of the direct psychiatric examination, the defendant was entirely apsychotic and demonstrated average intellectual capacities.

Conclusion: The board concluded that imputability was preserved, conditional on the court establishing that he actually committed the act. The assessment highlights that a chronic diagnosis of undifferentiated schizophrenia does not inherently result in a continuous state of non-imputability. Forensic accountability depends upon the specific cognitive and cognitive state tempore criminis, demonstrating that legal capacity remains intact when an individual functions without acute psychotic interference during the event.

Keywords: schizophrenia, imputability, tempore criminis.

64. Harmonization of Albanian Pharmaceutical Legislation with European Standards: Pharmacovigilance and the Safe Use of Psychotropic Medicines in Forensic Psychiatry

Author: Malvina Hoxha (Catholic University Our Lady of Good Counsel, Albania)

Background: The use of psychotropic medicines in forensic psychiatric settings raises important regulatory and patient-safety considerations, particularly regarding adverse drug reaction (ADR) monitoring, reporting obligations, and the management of psychotropic and controlled medicines.

Objective: This study examines the Albanian pharmaceutical legislative framework governing psychotropic medicines and pharmacovigilance and assesses its alignment with relevant European Union standards, with particular relevance to forensic psychiatric practice.

Methods: A qualitative comparative legal analysis was conducted between relevant Albanian pharmaceutical legislation and European Union provisions, particularly Directive 2001/83/EC and subsequent amendments concerning pharmacovigilance. The analysis focused on ADR reporting, pharmacovigilance responsibilities, safety monitoring, risk minimisation, and the regulatory management of psychotropic and controlled medicines.

Results: The analysis identifies substantial areas of convergence between Albanian and European regulatory frameworks, particularly regarding pharmacovigilance and the reporting and assessment of suspected ADRs. However, continued regulatory development and effective implementation remain important for strengthening medication safety and professional accountability. These issues are particularly relevant in forensic psychiatric settings, where psychotropic medicines may be administered within complex therapeutic and legal contexts.

Conclusions: Further harmonization of Albanian pharmaceutical legislation with European standards may strengthen the safe and accountable use of psychotropic medicines in forensic psychiatric care, while supporting patient protection, pharmacovigilance and regulatory accountability.

65. Нематеријалната Штета Основ за Можност за Манипулација при Судските Вештачења

Автори: Јасмина Степаноска, Тоде Вранишкоски (ГОБ „8-ми Септември“ - Скопје)

Кога медицинска докуметација не ја одразува реалноста: форензички предизвици во случај на наводна тешка телесна повреда.

По барање на Основен граѓански суд е извршено Супервештачење од трауматолози и психијатар, поради неусогласување на комбинираните вештања на трауматолози и психијатри од тужената страна и тужителот. Во доставената судска и медицинска докуметација според судско-медицинските квалификации се дијагностицирани Тешки телесни повреди и тоа: Fractura radii lateralis dextra et nasi cum edema localis. Contusio capitis et corporis. Syndroma Commotionale, Contusion region scapulatis et brachialis lateralis dexter. Erithema localis. Лекувањето било конзервативно со имобилизација на лева подлактица.

Двете претходни вештачења се извршени врз основа на доставена судска и медицинска документација без непосреден преглед на оштетената. На Супервештачењето беше повикана оштетената која уредно се јави и истата негираше дека дала полномошно за покренување на тужба за телесните повреди, дека не се здобила со тешки телесни повреди во предметната сообраќајна незгода, освен страв и набиеници во предел на носот, биле направени РТГ дијагностичкии следувања на глава, но е и на лева рака и негира дека носела имобилизација на лева рака.

Оштетената за време на егзаминацијата беше силно вознемирена, анксиозна, изненадена, пиша своерачна изјава на свој мајчин јазик исто така и вештите лица пишаа изјави во придружба на тројца случани сведоци присутни во Болницта и преведувач и истите беа потпишани од двете страни. Супервештачењето заедно со изјавите беше доставен до Основен граѓански суд е за понатамошно постапување.

При вештачења на нематеријална штета за телесни повреди медицинската докуметација да не се поистоветува автоматски со доказ за настанување на телесна повреда. Во отсуство на преглед, неопходна е автентичност и конзистентност на докуметцијата, медицинските наоди, нивната причинско-последична поврзаност со настанот и мултидисциплинарен форензички пристап.

Форензичкиот заклучок треба да се темели на интеграција на сите релеватни докази, јасно разграничување меѓу медицинскиот наод, неговото толкување и доказ.

Клучни зборови: тешки телесни повреди, непосреденп реглед, оштетена.

66. Fear, Acute Affective Reaction and Diminished Criminal Responsibility in a Life-Threatening Violent Event: A Forensic Psychiatric Case Report

Author: Dalibor Smilevski (Clinical Hospital Tetovo, North Macedonia)

Background: Assessment of criminal responsibility after an acute violent event is challenging when a previously healthy individual faces an unexpected life-threatening situation. Intense fear and emotional arousal may temporarily impair consciousness, judgment and behavioral control.

Case Presentation: A 37-year-old man with no previous psychiatric history was involved in a sudden firearm-related incident. Multiple rounds were fired at his vehicle, fatally injuring his close childhood friend and wounding the examinee. He experienced intense fear, panic, shock, anger and despair. During the incident, he took a firearm from the vehicle and fired toward the attacking vehicle.

Forensic Assessment: Psychiatric examination revealed preserved intellectual functioning, full orientation and no evidence of psychosis or persistent mental disorder. After the event, he reported severe anxiety, insomnia, nightmares, intrusive images and partial amnesia for parts of the incident. Reconstruction of his mental state indicated a high-intensity acute affective reaction to an objectively life-threatening situation. The sudden attack, witnessing his friend's death and sustaining injury were major precipitating factors. Intense fear, panic, anger and despair were associated with transient narrowing of consciousness, impaired rational judgment and reduced behavioral control. The reaction appeared immediate and unplanned, without evidence of prior preparation.

Conclusion: This case highlights the importance of reconstructing mental state at the precise time of the alleged offense. Integration of psychiatric and psychological findings, behavioral evidence and contextual factors is essential. In this case, the acute affective state substantially impaired rational judgment and behavioral control, resulting in substantially diminished criminal responsibility at the time of the offense.

Keywords: Criminal responsibility; acute affect; fear; narrowed consciousness; psychological shock.

67. Prolonged Grief Disorder Presenting as First-Time Offending Conduct: A Case Report

Authors: Nevena Nikolova, Adriana Bogdanovska Toskic, Maja Pendarovska (University Clinic of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Prolonged grief disorder (PGD) is associated with increased psychiatric morbidity, suicidal ideation and overt suicidality, indirect self-destructive behavior, and substance misuse, and its risk is elevated in survivors of violent loss. Indirect self-destructiveness, defined as prolonged engagement in potentially harmful behaviors while disregarding or being unaware of their adverse long-term consequences, includes norm-transgressing and risk-exposing actions. We present a case in which offending conduct may be understood in the context of PDG in an individual with no documented antisocial behaviour before bereavement.

Case report: A young man developed worsening of mood, intense yearning, inability to accept the death, preoccupation with the deceased, insomnia, nightmares, ruminative thinking, social withdrawal and suicidal ideation in the weeks following the violent death of his partner. He experienced recurrent intrusive images of her face and her voice calling him, at times in trance-like states in which intrusive content merged with reality. Psychopharmacological treatment led to no improvement. Approximately six months post-loss, he began regular cannabis use, which he described as the only thing that relieved the intrusive imagery. Around the time of the first anniversary of the death, he was arrested and subsequently convicted of possession of psychoactive substances for sale and of unauthorised possession of explosives; he later stated that the explosives had been acquired to end his life on the anniversary. He had no documented substance use, psychiatric treatment or contact with the justice system before the loss. A score of 56 on the Inventory of Complicated Grief (cut-off 25) was obtained two and a half years after the loss.

Conclusion: Prolonged grief may contribute to offending conduct by increasing indirect self-destructiveness, suicidality and substance misuse, which in turn raise exposure to high-risk situations and environments. Recognising this pathway may inform both clinical management of complicated bereavement and forensic assessment where offending follows a major loss.

Keywords: prolonged grief disorder, indirect self-destructiveness, substance misuse.

68. Reporting Of Peer Violence Among Secondary - School Students: Frequency And Reasons For Non-Reporting

Authors: Stojan Bajraktarov, Zylejha Salmani, Angela Ilievska, Sandra Ignova, Aleksandra Adjinikolova, Dehar Demiri, Naser Durmishi, Milos Milutinovik, Vaska Mitova (University Clinic Of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: A substantial proportion of adolescents who experience peer violence never disclose it to an adult; systematic data on reporting rates and barriers to help-seeking among secondary- school students in North Macedonia are lacking.

Aim: To determine the frequency of reporting peer violence among secondary-school students, to identify the most common reasons for non-reporting, and to estimate the prevalence of school bullying and cyberbullying.

Methods: Cross-sectional study using an anonymous 12-item questionnaire comprising a demographic section, the four standard HBSC (Olweus) bullying items and a reporting module. 884 students from secondary schools in Skopje, Gostivar, Bitola, Veles, Bogdanci and Tetovo (aged 14-19; 69.6% girls) participated.

Results: At least once in the past couple of months, 16.7% of students had been bullied at school, 22.1% cyberbullied, 16.4% had bullied others at school and 15.5% online; 29.8% had experienced at least one form. Boys were significantly more often perpetrators (cyber 22.4% vs 12.1%; p < 0.001), with no gender difference in victimisation. Among victims, only 28.4% (95% CI 22.9-34.5) had reported to an adult, 30.1% had told only a peer and 41.5% had told no one. The most frequent reasons for non-reporting were judging the incident not serious enough (27.3%) and believing that reporting would change nothing (21.1%); fear of retaliation ranked fourth (10.6%). Knowing whom to approach at school was the only independent predictor of reporting (OR = 0.38; 95% CI 0.17-0.89) and was significantly lower among victims (76.5% vs 87.6%; p < 0.001). Cyberbullying was not reported less often than school bullying.

Conclusion: Fewer than one third of bullied secondary-school students seek help from adults, mainly because they normalise the violence and distrust the effect of reporting rather than out of fear or ignorance.

Keywords: peer violence, bullying, cyberbullying, adolescents.

69. Beyond The Index Incident: The Role Of Developmental History In Adolescent Violence Risk Assessment

Authors: Evgenija Galeva, Biljana Gagachovska, Viktorija Korovesh, Aleksandra Coneva, Bojan Nikolovski (University Clinic Of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Violent behavior in adolescence should not be interpreted solely on the basis of an index incident. Childhood self-regulatory difficulties are associated with later externalizing and aggressive outcomes (Robson et al., 2020), while youth violence risk reflects interacting vulnerabilities across developmental domains (Herrenkohl et al., 2000; Mroczkowski et al., 2021). This case illustrates what developmental history can add to the forensic understanding of adolescent violence.

Case description: An 18-year-old male was admitted to a psychiatric unit following a violent interpersonal incident and police detention. Developmental history revealed persistent attentional difficulties, impulsivity, and impaired behavioral control from preschool age, with educational difficulties persisting across childhood. Early childhood was marked by exposure to domestic violence and direct physical victimization. During adolescence, substance use emerged alongside educational disengagement, loss of daily structure, and psychosocial deterioration.

Forensic relevance: Viewed in isolation, the index incident offered limited insight into the broader context of risk. Psychiatric assessment placed the violent behavior within a longer developmental trajectory of regulatory difficulties, cumulative adversity, and emerging dynamic risk factors. During police custody, escalating distress culminated in an impulsive suicidal act requiring emergency psychiatric hospitalization.

Assessment and management: Initial assessment showed affective distress, impulsivity, low frustration tolerance, and partial insight, without evidence of psychosis. During hospitalization, the patient became increasingly open about earlier experiences and demonstrated preserved emotional responsiveness and increasing therapeutic engagement. Treatment included stabilization, pharmacotherapy, and psychosocial interventions.

Conclusion: In adolescents, an index violent incident may be the first occasion on which longstanding developmental vulnerabilities become clinically and forensically visible. Developmental history does not excuse or deterministically explain violent behavior; rather, it provides clinically relevant context for forensic risk formulation. Its integration into psychiatric assessment may therefore inform risk management and targeted intervention.

Keywords: adolescent forensic psychiatry, developmental history, violence risk assessment, self-regulation, youth violence.

70. Born Dangerous or Made Violent? Delinquent Behaviour, Psychiatric Hospitalization and the Right to Responsibility in Adolescence

Authors: Biljana Gagachovska, Evgenija Galeva, Viktorija Korovesh, Aleksandra Coneva (University Clinic Of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Aggressive and delinquent behavior in children and adolescents represents a complex intersection between developmental psychopathology, child protection, social care, juvenile justice, and forensic psychiatry. The dilemma becomes particularly pronounced among young people raised in residential institutions, foster care, dysfunctional families, and other high-risk environments, where developmental trauma, neglect, disrupted attachment, inconsistent caregiving, and repeated experiences of abandonment are common. These vulnerabilities require recognition, protection, and treatment when clinically indicated. They do not, however, transform every act of aggression, serious rule violation, or delinquency into psychiatric illness.

A concerning systemic pattern emerges when psychiatric services become the default destination for young people whose behavior other institutions are unable, unwilling, or insufficiently equipped to contain. Following aggression, destruction, repeated rule-breaking, or criminal acts, psychiatric hospitalization may become an apparently convenient solution—not because an acute mental disorder requiring inpatient treatment has been established, but because the surrounding system has failed to provide consistent care, structure, boundaries, supervision, and consequences.

In this context, psychiatry risks becoming the endpoint of failures occurring elsewhere in the system. Instead of undertaking the more demanding task of caring for, containing, educating, setting boundaries, and teaching a young person to assume responsibility, the system may choose the seemingly easier task of defining and treating the young person as a psychiatric patient. In doing so, institutional and systemic failures risk being relocated into the child and reframed as individual psychopathology.

Suicidal statements further complicate this process. Every expression of suicidality requires careful and competent risk assessment; however, suicidal communication does not automatically constitute an indication for psychiatric hospitalization. Within recurrent externalizing or delinquent behavioral patterns, some young people may learn that escalation, threats, or psychiatric symptom presentation can interrupt anticipated consequences and transfer responsibility from the original setting to psychiatric services. If hospitalization repeatedly becomes the predictable outcome of such situations without clear clinical indication, the system itself may inadvertently reinforce externalization of responsibility and avoidance of consequences.

This is developmentally significant. Adolescence is a critical period for learning the relationship between choice, action, consequence, and responsibility. Developmentally appropriate accountability should therefore not be understood merely as punishment. When combined with protection, stable caregiving, psychosocial intervention, and proportionate consequences, it represents an essential developmental experience through which agency, self-regulation, empathy, boundaries, and responsibility toward others can be internalized.

Forensic child and adolescent psychiatry must consequently maintain clear boundaries between explanation and exculpation, vulnerability and incapacity, distress and mental disorder, treatment and behavioral containment. Its task is neither to punish vulnerable young people nor to absolve them indiscriminately, but to determine when psychopathology genuinely compromises relevant capacities and when, despite adversity, a young person remains capable of understanding, choice, and responsibility.

A functional child-protection and juvenile justice system must be capable of holding two truths simultaneously: a child may have been profoundly failed by adults and institutions, and that same child may still need to learn to take responsibility for his or her actions. Protection and accountability are not opposites. For vulnerable young people, both may be essential components of development. Perhaps, therefore, one of the most neglected rights of high-risk adolescents is the right to responsibility—the right not to have every unacceptable behavior medicalized, the right to encounter coherent boundaries and meaningful consequences, and the right to be regarded as capable of learning from one's actions rather than being indefinitely positioned as either a victim or a psychiatric patient.

Trauma requires understanding. Mental disorder requires treatment. But aggression and delinquency alone require neither a psychiatric diagnosis nor a hospital bed.

Keywords: juvenile delinquency; adolescent aggression; developmental trauma; accountability; psychiatric hospitalization; forensic child and adolescent psychiatry; child protection; juvenile justice.

71. Parental Consent, Adolescent Dissent: A Case-Based Medico-Legal Analysis Of Psychiatric Hospitalization Of A Minor

Authors: Viktorija Korovesh, Biljana Gagachovska, Evgenija Galeva, Marija Ruvcheska (University Clinic Of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Psychiatric hospitalization of minors raises complex questions regarding parental authority, adolescent autonomy, decision-making capacity, and restriction of liberty. A particular medico-legal dilemma arises when a parent consents to hospitalization while an adolescent explicitly refuses to remain in hospital.

Case description: A 17-year-old male was admitted for his first psychiatric hospitalization due to an acute psychotic state characterized by marked suspiciousness, hostility, paranoid-interpretative delusional ideas, aggression, and significant functional impairment. He showed poor insight, denied the need for treatment, repeatedly requested to return home, and actively opposed hospitalization, while his father provided consent. During the first days of admission, he attempted to leave the psychiatric ward.

Forensic relevance: The case highlights the distinction between parental authorization for treatment and restriction of an adolescent's liberty. North Macedonian legislation provides a legal role for parents or legal representatives in medical decision-making for minors, while psychiatric admission and restriction of freedom of movement are additionally regulated by mental health and procedural legislation. Persistent adolescent dissent therefore raises the question of when hospitalization authorized by a parent may function as *de facto* involuntary hospitalization and which procedural safeguards should apply.

Assessment and management: Continued inpatient treatment was considered clinically necessary and antipsychotic and anxiolytic treatment was initiated. As psychotic symptoms subsided over the following 7–10 days, insight improved and the patient's attitude toward hospitalization changed. He became accepting of inpatient treatment, expressed fear related to his experiences, and acknowledged a need for help.

Conclusion: Parental consent and clinical necessity do not alone resolve the question of voluntariness when an older adolescent actively opposes hospitalization. This case highlights the importance of repeated assessment of adolescent dissent, decision-making abilities, psychopathology, insight, risk, proportionality, and the legal basis for restriction of liberty.

Keywords: adolescent forensic psychiatry, parental consent, adolescent dissent, involuntary hospitalization, decision-making capacity.

72. Beyond Iq: Intellectual Disability, Behavioral Control And Criminal Responsibility In Adolescence - A Case Report

Authors: Marija Ruvcheska, Ana Madikj Kostovski, Viktorija Korovesh, Biljana Gagachovska, Evgenija Galeva (University Clinic Of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Assessing criminal responsibility in adolescents with intellectual disability requires careful differentiation between cognitive limitations, developmental immaturity and the specific capacities to understand and control the conduct under examination. This case illustrates the forensic significance of substantially impaired, yet partially preserved, capacities and their dependence on environmental structure.

Case presentation: A 17-year-old male underwent a multidisciplinary forensic psychiatric and psychological assessment following recurrent harmful sexual behavior involving family members and repeated psychiatric hospitalizations. Evaluation integrated direct examination, maternal collateral history, psychological testing and previous inpatient records. Findings supported mild intellectual disability, with an estimated IQ range of 51–62, longstanding adaptive difficulties and comorbid epilepsy. Clinical features included concrete thinking, emotional immaturity, impaired social judgement, limited appreciation of interpersonal boundaries and markedly compromised impulse control. No evidence of active psychosis was identified. He acknowledged aspects of the reported behavior but minimized its seriousness and demonstrated limited understanding of its consequences. Behavioral inhibition was better preserved during structured assessment than in the home environment.

Forensic formulation: The assessment concluded that his capacities to understand the significance of his actions and to control his behavior were substantially reduced, although not abolished. Better inhibition under external supervision suggested residual regulatory capacity; it did not establish equivalent self-control in less structured circumstances. The formulation identified an ongoing risk of recurrence and a need for sustained multidisciplinary treatment, structured supervision and protection of vulnerable family members.

Conclusion: This case highlights the distinction between residual capacity and adequately preserved autonomous functioning. Neither intellectual disability alone nor the absence of psychosis resolves the question of criminal responsibility. An individualized forensic opinion must specify the nature, degree and contextual variability of cognitive and volitional impairment in relation to the conduct under examination, while distinguishing clinical findings from the ultimate legal determination.

Keywords: intellectual disability; adolescent forensic psychiatry; criminal responsibility; inhibitory control; harmful sexual behavior.

73. Clinical-Psychiatric and Histopathological Correlations in Cases of Sudden Death in a Psychiatric Hospital

Authors: Gligor Ristovski, Ana Ivcheva, Ljupcho Chakar, Rosica Chakar, Goran Pavlovski, Viktorija Belakaposka Srpanova, Natasha Bitoljanu, Zlatko Jakjovski, Aleksandar Stankov (Institute for forensic medicine, criminalistics, and medical deontology, Medical Faculty, Skopje, University of "Ss Cyril and Methodius" – Skopje, North Macedonia)

Introduction: Sudden death in psychiatric institutions represents a complex medical and forensic challenge. Risk factors include long-term psychopharmacological therapy, reduced ability to verbalize somatic symptoms, and undiagnosed somatic diseases. Histopathological analysis of autopsy organs helps identify the pathological substrate of death.

Aim: 1) To systematize histopathological changes in the central nervous system, lungs, liver, and kidneys described in sudden death in psychiatric institutions; 2) To demonstrate the contribution of histopathological analysis through autopsy findings.

Material and Methods: A combined methodology was applied: a narrative literature review and retrospective qualitative analysis of histopathological specimens, correlating clinical-psychiatric data with tissue changes.

Results: Reported findings include cerebral edema and anoxic-ischemic injury in the central nervous system; aspiration pneumonia and embolism in the lungs; steatosis and cirrhotic changes with portal hypertension in the liver; and acute tubular necrosis and interstitial nephritis in the kidneys. Histopathological analysis of autopsy material enabled identification of findings relevant to the cause and mechanism of death. This is illustrated by 20 histopathological microphotographs. The presentation focuses exclusively on histopathological findings, without psychiatric diagnoses or personal identities.

Conclusion: Histopathological evaluation contributes to determining the morphological substrate of sudden death in hospitalized psychiatric patients. Identification and verification of tissue changes support integration of clinical-psychiatric presentation with somatic pathology and autopsy findings, enabling objective morphological assessment and contributing to medico-legal certainty.

Keywords: histopathology, sudden death, psychiatric hospital, autopsy, forensic pathology.

74. Postoperative Delirium and Acute Psychiatric Presentations: Diagnostic and Forensic Dilemmas

Authors: Frosina Nikolikj, Dafina Karadjova, Atanas Sivevski (PHI University Clinic of Gynecology & Obstetrics – Skopje), Slavica Arsova, Stefanija Mitrovska, Hristijan Hadzi-Angjelkovski (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia), Krsto Nikolikj (PHI University Clinic for Surgical Diseases "St. Naum of Ohrid" – Skopje, North Macedonia)

Background: Postoperative delirium is one of the most common complications in older surgical patients, yet it is still often missed or mislabelled. The hyperactive form can look like primary psychosis or acute agitation. The hypoactive form is easily mistaken for depression, fatigue or residual sedation. These errors have clinical consequences, and sometimes legal ones.

Aim: To review the pitfalls in distinguishing postoperative delirium from other acute psychiatric presentations, and to outline the forensic questions that arise when a patient's mental state is impaired after surgery.

Methods: A narrative review of literature published between [2015] and [2025], identified through PubMed and [Scopus]. The review included current ESAIC guidance and the consensus nomenclature for perioperative neurocognitive disorders.

Results and Discussion: Diagnosis rests on DSM-5 criteria: an acute, fluctuating disturbance of attention and awareness, with additional cognitive change, attributable to a medical cause. Validated tools such as the 4AT on the ward and the CAM-ICU in intensive care improve detection. Because symptoms fluctuate, these tools are only reliable when repeated.

The differential diagnosis includes: emergence agitation, alcohol or benzodiazepine withdrawal, anticholinergic toxicity, serotonin syndrome, non-convulsive status epilepticus, less often, a first episode of primary psychiatric illness. Managing hyperactive delirium as a psychiatric emergency without looking for its cause can delay treatment of sepsis, hypoxaemia or metabolic derangement.

The forensic problems come from the same features: (1)Capacity: a fluctuating mental state makes capacity to consent to or refuse treatment hard to judge at the time and even harder to reconstruct later. (2)Restrictive measures: physical restraint and sedation raise questions of proportionality and consent. (3)Liability: self-extubation, falls and aggression towards staff can lead to claims. (4)Reliability of statements: complaints or allegations made during delirium may later be treated as accurate accounts.

Structured, time-stamped documentation is the clinician's main protection. It should cover screening results, capacity assessment and the reason for every restrictive measure. [In our institution, e.g. routine 4AT screening in the recovery unit / a joint protocol with the psychiatry service] has [e.g. reduced unplanned psychiatric transfers].

Conclusion: Postoperative delirium should be treated first as an acute medical problem and only then as a psychiatric presentation. Routine screening, a systematic differential diagnosis and careful documentation, supported by early collaboration between anaesthesiology and psychiatry, improve patient outcomes and reduce medicolegal risk.

Keywords: postoperative delirium; perioperative neurocognitive disorders; 4AT; CAM-ICU; decision-making capacity; informed consent; physical restraint; forensic psychiatry; anaesthesiology.

75. Neurodevelopmental Dysregulation vs. Forensic Intent: Differentiating Ego-Dystonic OCD Compulsions from Unlawful Conduct in Adult ASD

Authors: Iva Velichkovska (Neurodevelopmental Center “In My world”, Skopje, North Macedonia)

Background: Individuals diagnosed with Autism Spectrum Disorder (ASD) and comorbid Obsessive - Compulsive Disorder (OCD) frequently engage in structured behaviors to manage cognitive rigidity. When these compulsions involve photographing unfamiliar individuals in public, they are rapidly misattributed as criminal voyeurism or deliberate sexual misconduct, leading to inappropriate criminalization rather than targeted clinical intervention.

Objectives: This case study illustrates the clinical assessment of a 25-year-old male diagnosed with ASD presenting with compulsive public photography of women, establishing differential diagnostic criteria between OCD fixations and forensic intent (mens rea).

Methods: A qualitative clinical analysis was conducted using a neurodiversity-sensitive assessment matrix, integrating MMPI-2 testing, indirect clinical interviewing, collateral caregiver data, Functional Behavioral Analysis (ABC model), Visual Analog Scales, and Theory of Mind testing (RMET). Assessment evaluated functional behavior drivers, sensory-perceptual OCD fixations, and awareness of implicit privacy boundaries.

Results: The evaluation confirmed an ego-dystonic OCD compulsion driven by anxiety reduction, entirely devoid of voyeuristic arousal or forensic intent (mens rea). Elevated psychasthenia (Pt) on MMPI-2 and severe RMET deficits corroborated the diagnosis. Adapted ERP therapy and boundary psychoeducation yielded initial compulsion neutralization, though full clinical extinction remains ongoing given the required longitudinal trajectory for individuals diagnosed with ASD.

Conclusion: Accurate differential diagnosis between OCD-mediated compulsions and actual unlawful conduct is critical to prevent unjust legal trajectories and provide human-rights-oriented care.

76. When A Clinical Report May Carry Legal Weight: Forensic And Ethical Boundaries In Documenting Intimate Partner Violence

Authors: Aleksandra Miseva, Sanja Vasovikj, Nikola Dimov, Angela Madzarova (JZU Opsta Bolnica Strumica, North Macedonia)

Background: A psychiatric report written for clinical care may later enter legal proceedings. In intimate partner violence, psychiatrists must distinguish patient reports from clinical findings and avoid conclusions about unassessed individuals.

Case description: A woman in her sixties presented with anxiety and tearfulness. She reported years of humiliation, threats, psychological abuse, and physical assaults by her husband, along with his daily alcohol use and intoxication. Although she had reported the violence to the police, she feared further aggression and felt unsafe at home. On examination, she was anxious, with reactive affect, but fully oriented and cooperative; rapport was readily established. Adjustment disorder was diagnosed (ICD-10 F43.2).

Forensic relevance: The report later entered court proceedings; the team learned that a court had ordered six months of treatment for the husband at a specialist psychiatric hospital. The report separated her account from clinical findings and recommendations. As the husband had not been assessed, her statements could not establish alcohol dependence. The psychiatrist could record safety concerns but could not establish allegations as fact, determine legal responsibility, assess his risk, or predict treatment outcome.

Assessment and management: Treatment for anxiety and stress-related symptoms was started. She was advised to contact the police and social services after any further threat or assault. Documentation focused on her mental state and safety. Written informed consent was obtained.

Conclusion: Patient safety and procedural fairness are complementary aims. Both require documentation that identifies information sources and the limits of clinical opinion. Reports should communicate safety concerns without presenting allegations as facts, diagnosing an unassessed person, or replacing an independent forensic assessment.

Keywords: intimate partner violence, clinical documentation, forensic ethics.

77. Anxiety In A Forensic Psychiatric Inpatient Setting

Authors: Vilma Videnova, Olivera Subotin Popovska, Sashka Simonovska Petrovska (Psychiatric Hospital Skopje - Skopje), Nensi Manusheva (University Clinic Of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: A high proportion of patients in forensic psychiatric facilities are diagnosed with comorbidity, most often with schizophrenia, paranoid psychosis, organic brain syndrome, various personality disorders and comorbid substance abuse. Anxiety has often been under recognized and underdiagnosed in forensic psychiatric inpatients. Causes and triggers for anxiety are institutional environment, limitation of personal freedom and social connection, dual stigma (severe mental illness and offender identity), safety tensions between clinical care and security measures. Prior exposure to violent incidents and trauma amplify anxiety and fear.

Objective: The aim of this study is to analyze comorbid anxiety in hospitalized forensic inpatients.

Methods: 28 hospitalized forensic male patients aged 19-65 were examined in terms of anxiety. They completed self-administered questionnaire Zung Self-Rating Anxiety Scale (SAS). The scale consists of 20 items, each of which is scored on a four-point scale and aims to measure the severity of anxiety symptoms in individuals.

Results: Comorbid anxiety was found in 70 % of the patients based on psychiatric examination and Zung Self-Rating Anxiety Scale (SAS) Score. 30% of the patients were in a normal range, 50% had mild to moderate anxiety, 15 % had marked to severe anxiety, and 5% had extreme anxiety.

Conclusion: Comorbid anxiety is present among forensic psychiatric inpatients and it leads to poorer medication compliance, poorer adjustment and treatment response and increases the likelihood of violent behaviour. This affects the treatment outcome and quality of life of these patients. Management and interventions that may reduce anxiety include environmental modifications, psychosocial approaches and pharmacological support to manage severe anxiety and impulsivity.

78. Women With Mental Illness: Cruelty A Ground For Divorce, Not Mental Illness

Author: Indira Sharma (Professor Emeritus, National Academy of Medical Science, New Delhi, India)

Background: Although Hind Marriage and Special Marriage Acts in India provide for divorce on the ground of mental illness, petitions for divorce from husbands have been on the ground of cruelty, even when the real ground was mental illness.

Aim: To determine whether complainants succeeded in getting divorce on cruelty? Whether courts were able to understand that the real ground is mental illness. Did courts order inquiry into the presence of mental illness and treatment of the same.

Method: Judgements of cases of divorce pleading for divorce on the ground of cruelty shall be retrieved and studied. Cases with high probability of mental illness shall be examined for the Court intervention/Judgement.

Results: In many cases divorced was granted on the ground of cruelty, without looking into the possibility of underlying mental illness. Eg false charge of unchastity, attempt to commit suicide, repeated rebukes,abuses, nagging, taunts, curses and compulsions for apologising needlessly by husband, could be because of an underlying mental illness.

Conclusion: Courts need to appreciate the possibility on underlying mental illness, which should be treated. This would uphold the right to treatment and also possibly save many marriages.

Key words: Mental Illness, Marriage, Cruelty, Divorce.

79. Алтернативни Казнени Мерки кај Шизофрениот Пациент

Author: Ѓорѓе Хаџи-Ангелковски

Шизофрениот пациент (веќе дијагностициран) ретко врши кривични дела, кои се движат во рамките на крвни деликти до помали кражби поради егзистенцијални причини. Еден отсто од популацијата врши околу 60 отсто од севкупниот криминал, повторувачки криминал, каде популацијата на шизофренија е незначителна. Но виктимизацијата на шизофрениот пациент е висока, пет пати поголема од останатата популација, тие се подложени на злоупотреба и насилство од секаков вид.

Крвните деликти предизвикани од пациенти со шизофренија се драстични, се одвиваат во семејни услови кога страда некој член на семејството, предизвикуваат вознемиреност кај јавноста со негативна сензибилизација и предубедувања кон севкупната популација на шизофренија, која се доживува како виолентна и непредвидлива.

Законот превидува два типа на мерки за безбедност – задолжително чување и лекување во психијатриска установа и задолжително лекување од слобода. Во наши услови на скромни сервиси на ментално здравје превалира првата мерка, со чести случаи на деценски престој на шизофрениот пациент во ментални установи, каде можностите за рехабилитација и ресоцијализација се минимални. Ова значи дека инкарцерацијата наспроти третман и инклузија преовладува како принуден, но нехуман начин, психијатарот стравува да даде предлог за замена на мерката поради страв од повторување во услови на слаба психијатриска грижа во заедницата.

Во трудот ќе се дискутира околу можостите за дополнителни мерки и начини за ресоцијализација и инклузија на оваа популација.

80. Форензични Аспекти на Граничната Личност

Author: Ѓорѓе Хаџи-Ангелковски

Граничната структура на личност е чест предмет на кривичниот прогон, бидејќи е голем процентот на сторители кај оваа популација (преку 40 отсто), за разлика од 1 отсто кај севкупната популација, која ќе стори сериозен и повторувачки криминал. Сложените интерперсонални односи во кои запаѓаат, со обид за контрола на објектот на интерес, преплавеноста со негативни афекти и слабата импулсна контрола се најчеста причина за сторување на дела од страст.

Најчест интерес на судот е пресметливоста на овие личности во текот на сторување на делото. Психијатарот откако ја констатирал оваа дијагностичка категорија, дека не се работи за асоцијална структура, каде владее експлоатацијата и недостаток на емпатија на другиот, треба да обрне внимание на развојните аспекти на граничната личност, особено во раните години кога тој најчесто бил жртва на семејно насилство и сексуална злоупотреба (преку 90 отсто биле објект на некој тип на злоставување).

Исто така потребно е да се разгледаат и комплицираните социјални односи кои ја разнишуваат нивната вулнерабилна структура, деструктивните репетативни образци на поведение, честата злоупотреба на ПАС за да се добие објективната слика во разјаснувањето на кривичното дело. Низ неколку примери од секојдневната пракса ќе се направи обид за појасен пристап кон оваа проблематика.

81. Bipolar Affective Disorder Vs. Paranoid Schizophrenia In The Process Of Determining Ability To Work, A Case Report From A Forensic Psychiatric-Psychological Evaluation

Authors: Maja Spirova, Zoja Babinkostova, Bojana Gjorgovska, Maja Petrusevska, Aleksandar Paketciev (University Clinic of Psychiatry, Faculty Of Medicine, Ss. Cyril And Methodius University In Skopje, Skopje, North Macedonia)

Background: Bipolar BAD is characterized by recurring episodes involving disturbances in mood and behavior as elevated mood and increased psychomotor activity, while in others involve depressive changes. The frequency of episodes and the pattern of remissions vary. Both phases can be accompanied by psychotic symptoms. PS on the other hand is a disorder characterized by paranoid delusions and hallucinations, and it follows a progressive course.

Case description: a psychiatric and psychological expert assessment initiated in the context of ongoing court proceedings. The matter arises from a decision by the competent pension insurance fund of the RSM, which denied the individual's claim for a disability pension. The authorized representative submitted a request for a psychiatric and psychological evaluation of the individual's work ability, stating that the person has suffered from PS for 20years and is incapable of work. The purpose of the request is to determine individual can be considered capable for work.

Forensic relevance: A review of the medical documentation was conducted, indicating that the individual had received treatment for the following diagnoses: F20.0 F23.9 F41.2 F31.1 F31.7 and F41.3.

Assessment and management: A psychiatric-psychological assessment was conducted, establishing a diagnosis of BAD. The condition manifests with depressive symptoms of mild to moderate intensity (F31.3 according to ICD-10).

Conclusion: In the case of BAD, it is clear that professional functioning is impaired during manifest psychotic symptomatology, however this impairment is not permanent, that is upon the resolution of psychotic symptoms and the onset of remission,professional functionality is restored, resulting in relatively normal professional functioning. The symptoms of PS on the other hand are never absent. They result in persistent impairment of professional functioning due to recurrent exacerbations, residual psychotic symptoms, cognitive deficit, impaired social and interpersonal functioning.

82. Beyond the Battlefield: Resilience or PTSD? Psychological Consequences of Working in a War Zone – A Case Series

Authors: Verica Malinkova Dimovska, Kamelija Simeonova (Clinical Hospital Acibadem Sistina - Skopje)

Background: PTSD is a recognized consequence of armed conflict. While direct combat and life-threatening events are established risk factors, prolonged occupational exposure may also affect non-combat personnel.

Case Presentation: We present three patients with PTSD following exposure to a war-zone environment. The first patient, a 48 years old male from N.Macedonia was exposed to a work-related explosion at a military base and sustained serious physical injuries. Following medical stabilization, he developed intrusive memories, nightmares, avoidance, hyperarousal, irritability, sleep disturbance, and depressive symptoms. Treatment with sertraline 150 mg/day and risperidone 1 mg/day, together with supportive psychotherapy, resulted in improvement; he is currently physically and psychiatrically stable. The other two cases involved male Kosovo citizens, aged 48 and 45 years, working for approximately two years in a military base in Gaza in administrative and logistical positions. Despite not being directly involved in combat, both developed sleep disturbances, nightmares, increased anxiety, and mild irritability, particularly at home. Both received antidepressant treatment and psychiatric follow-up.

Discussion and Conclusion: These cases illustrate that the consequences of war extend beyond direct combat exposure. The first case reflects an acute life-threatening traumatic event, while the other two demonstrate the potential impact of prolonged occupational exposure without direct combat involvement. Separation from family, chronic uncertainty, repeated stressful events, and difficulties transitioning between war-zone and home environments may contribute to symptoms. Early recognition, preventive support, and timely psychiatric treatment are important for personnel working in conflict-affected environments.

Keywords: PTSD, war zone, occupational exposure, Gaza, explosion, resilience, trauma, anxiety.

83. Treatment Deviations And Diagnostic Non-Confirmation In Trauma-Related Compensation Claims Among Military-Zone Contractors: A Multimodal Independent Forensic Reassessment (2020–2023)

Author: Vera Jovanovska-Tipko (PZU Ekvilibrium, Skopje, North Macedonia)

Background: Claimants formerly deployed to conflict zones as private contractors, referred for independent medical examination (IME), present with a pre-existing diagnostic label of post-traumatic stress disorder (PTSD) and documented treatment history. Multimodal forensic reassessment allows examination of diagnostic validity and the adequacy of treatment delivered before referral.

Objective: This study examined, among claimants, (1) the frequency of treatment deviations from recommended practice before IME referral; (2) the temporal relationship between treatment initiation and legal-proceeding commencement; and (3) the rate of diagnostic confirmation of pre-existing PTSD on reassessment.

Methods: We retrospectively reviewed 103 independent forensic psychiatric evaluation reports, including clinical interview, mental status examination, and psychometric testing such as MMPI validity indices were administered. Pre-referral documentation was reviewed for pharmacological appropriateness, provision of trauma-focused psychotherapy, and evaluator-documented treatment deviations. Temporal proximity between treatment initiation and legal-proceeding commencement was recorded. Diagnostic conclusions followed DSM-5/ICD-10 criteria and, where administered, instruments including the CAPS-5 and PCL-5; symptom validity was independently assessed using the Structured Inventory of Malingered Symptomatology (SIMS; cut-off >14).

Results: Treatment deviations from recommended practice were identified in 51.5% of cases, most commonly prolonged benzodiazepine prescribing (29.1%), limited trauma-focused psychotherapy (28.2%), and overall treatment or pharmacotherapy left unrevised despite documented lack of clinical improvement (35.9%). Treatment onset occurred in temporal proximity to legal-proceeding commencement in 85.4% of cases. Pre-existing PTSD was not confirmed in 94.2% of cases (73.5% of SIMS-tested cases scored above cut-off). Non-confirmation and a treatment deviation co-occurred in 49.5% of cases.

Conclusion: In this sample, pre-existing PTSD diagnoses were infrequently confirmed, while treatment deviations and legal-timing associations were common — findings that support the value of multimodal forensic reassessment for diagnostic accuracy and evaluation of treatment adequacy. The small confirmed-diagnosis subgroup precluded formal statistical comparison and warrants replication in larger, multi-site samples.

Keywords: PTSD, independent medical examination, forensic psychiatry.

84. Typology Of Stalking Offenders And The Significance Of Forensic Psychiatric Assessment

Authors: Snezana Djordjevic, Stanislava Kijac (SBPB Kovin, Serbia)

Background: Stalking is a complex and heterogeneous form of interpersonal violence that encompasses a broad spectrum of persistent, intrusive, and unwanted behaviors directed toward a specific individual. Beyond its legal implications, stalking represents a significant public health and forensic concern due to repeated victimization and the potential escalation to serious violence. Within this context, forensic psychiatric evaluation plays a central role in determining criminal responsibility and informing judicial decision-making.

Objective: This paper aims to critically examine the principal typologies of stalking offenders and to evaluate the relevance of forensic psychiatric assessment in determining criminal responsibility, with particular emphasis on the interaction between psychopathology, motivational factors, and legal standards.

Methods: The analysis synthesizes current theoretical models of stalking offender typology and examines their implications for forensic psychiatric practice, with a particular focus on the assessment of cognitive and volitional capacities at the time of the offense.

Results: Evidence suggests that stalking offenders constitute a highly heterogeneous population differing in motivation, psychological functioning, personality organization, and psychiatric morbidity. The integration of offender typology with comprehensive forensic psychiatric assessment enhances the accuracy of criminal responsibility evaluations, improves structured violence risk assessment and supports individualized case management.

Conclusions: The determination of criminal responsibility in stalking cases requires an individualized, evidence-based multidisciplinary approach that integrates criminology, psychiatric, psychological, and legal perspectives.

Keywords: stalking; offender typology; forensic psychiatry; criminal responsibility.

85. Institutionalization as a Factor in the Assessment of Legal Capacity

Authors: Bojan Nikolovski, Aleksandar Kjosev (PHI General Hospital Strumica), Petar Sotiroski, Antoni Novotni, Ivana Sajkovska, Evgenija Galeva (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia)

Prolonged psychiatric hospitalization may have significant consequences beyond the treatment of the underlying mental disorder. Long-term institutionalization can contribute to the development of hospitalism, characterized by reduced initiative, diminished autonomy, social withdrawal, and loss of adaptive skills, potentially resulting in substantial impairment of independent functioning. We present the case of a 65-year-old patient with a diagnosis of bipolar affective disorder, with multiple and prolonged hospitalizations over the previous 12 years. At the time of the assessment, the patient was undergoing his 34th psychiatric hospitalization. A forensic assessment was conducted to evaluate his condition legal capacity, along with consideration of the need for the appointment of a legal guardian to represent him in legal proceedings, and protect his financial and property interests. The psychiatric examination, and the review of the available medical documentation, demonstrated a chronic course of the psychiatric disorder, significant impairment of psychosocial functioning, and marked dependence on the institutional environment. The patient exhibited features consistent with institutionalization and limited ability to assume responsibility for his affairs. It was concluded that the chronic course of the psychiatric disorder, together with the developed institutionalization and significant impairment of independent functioning, substantially affected the patient's ability to independently protect his rights and interests. The patient was assessed as lacking legal capacity. This case illustrates the importance of institutionalization as a potential factor contributing to functional impairment in chronic psychiatric disorders. It also highlights the importance of developing community-based and extrahospital psychiatric services, with inpatient treatment being provided for the shortest clinically appropriate duration.

86. Psychological and Psychiatric Assessment of Criminal Responsibility in a Person with Bipolar Disorder: a Case Report

Authors: Marijana Stankovic, Slavica Arsova Hadji Angjelkovska, Zoja Babinkostova (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia)

Introduction: Forensic psychiatry and psychology require a multidisciplinary approach in the medico-legal evaluation of individuals with severe mental disorders who have committed criminal offenses. The assessment of sanity at the time of the offense is a key element that determines criminal responsibility and subsequent medico-legal treatment.

Objectives: The aim of this paper is to illustrate the complexity of forensic psychiatric and psychological diagnostics in a subject with bipolar disorder who has committed a criminal offense.

Case Report: The case of a 50-year-old male is presented, who was brought in for forensic psychiatric and psychological observation and diagnostics following a criminal offense (endangering security) committed against a neighbor.

Psychiatric Findings: The subject manifests a long-standing history of psychotic symptomatology due to an untreated condition. During the period of the offense, he was under the influence of paranoid-hallucinatory symptomatology that governed his actions, rendering him unable to comprehend the meaning and consequences of his act.

Psychological Testing: A standard forensic psychological battery of tests was applied (Progressive Matrices, MMPI-2, Human Figure Drawing, Rorschach Test, clinical interviews). The results indicate severe disintegration of the thinking process, impairment of reality testing, as well as a complete loss of the capacity for reasoning and controlling one's actions.

Discussion: The correlation between mania with psychotic symptomatology and the motive for the criminal offense is direct.

Conclusion: The findings emphasize that in such patients, the focus should be directed toward mandatory medico-legal treatment in secure psychiatric facilities, rather than punitive retribution, in order to reduce future risks.

87. Neuropsychological Assessment in an Elderly Individual and the Right to Property Conveyance: A Case Report

Authors: Marijana Stankovic, Stefanija Mitrovska, Marija Bojadzi, Ivo Kunovski, Kalina Bojkovska (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia), Maja Arsov (Psychiatric Hospital Skopje), Frosina Nikolikj (University Clinic of Gynecology) (University Clinic of Gynecology, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia)

Introduction: In modern forensic-psychiatric practice, the assessment of legal capacity and decision-making ability in elderly individuals represents a complex and responsible challenge. With the increase in life expectancy, we increasingly encounter cases where older individuals with mild cognitive impairment or early stages of neurodegenerative diseases perform legal transactions, such as property transfer or gifting. The aim of this paper is to present the role and significance of comprehensive neuropsychological assessment in determining mental state and legal capacity in such contested legal contexts.

Case Report: The paper presents a forensic-psychiatric and detailed neuropsychological examination of a 76-year-old subject, whose capacity for legally valid property disposition was contested due to a diagnosis of Alzheimer's disease. The clinical evaluation included a psychiatric-psychological interview, as well as a standardized neuropsychological test battery (MMSE, ACE-R, RAVLT, TMT-A, TMT-B, Clock Drawing Test, BNT). The results showed the presence of moderate cognitive deficits that directly affected criticality, decision-making, and susceptibility to external influences during the critical period in an attempt to conclude the legal act.

Discussion: The case illustrates the tension between the individual's right to self-determination and the need to protect vulnerable elderly individuals from manipulation and financial exploitation. Neuropsychological testing enabled the objectification of the cognitive status that went beyond normal aging, which was the key element in making an objective expert finding and opinion.

Conclusion: Precise neuropsychological assessment represents an essential instrument in forensic psychiatry. It provides scientifically grounded arguments for assessing the validity of legal acts enacted by elderly individuals, contributing to fairness and legal certainty.

88. Ethical Aspects of Responsible Public Reporting of Psychiatric Evidence During Active Legal Proceedings: A Focused Evidence Review

Authors: Stefanija Mitrovska, Slavica Arsova, Meri Jordanovska, Adriana B. Toskikj, Simona K. Kochoska, Vaska D. Mitova, Nevena Nikolova, Dora Kamnar, Hristijan Hadzi-Angjelkovski (University Clinic of Psychiatry, Medical Faculty, Ss.Cyril and Methodius University, Skopje, North Macedonia), Frosina Nikolikj (PHI University Clinic of Gynecology & Obstetrics – Skopje), Krsto Nikolikj (PHI University Clinic for Surgical Diseases "St. Naum of Ohrid" – Skopje)

Introduction: Ethical standards for media reporting on suicide are comparatively mature and internationally codified, yet a distinct problem persists alongside them: once a case carries a possible psychiatric or psychological dimension, media coverage and public commentary often move well beyond confirmed fact, speculating on the individual's mental state, assigning moral blame, and exposing intimate personal details - with no comparable governance framework constraining this. A 2026 North Macedonian trial concerning a domestic-violence-induced suicide of a mother and daughter - covered extensively over several months, and accompanied by widespread public speculation about the victim's psychological state and moralizing commentary on her private life, despite no confirmed psychiatric diagnosis or treatment - prompted this review.

Objectives: To synthesize evidence on (1) media framing of psychiatric illness and psychological causation in relation to violence and crime, and (2) the international governance of public commentary on individuals' psychiatric or psychological state - whether clinically confirmed or merely speculated - during active legal proceedings, and to characterize the resulting regulatory gap.

Methods: A focused review synthesized peer-reviewed studies and institutional guidance identified through PubMed and targeted supplementary searches (WHO/IASP, WPA, AAPL, national media-regulatory publications), spanning 1998–2026. Eligible sources addressed content-analytic study of psychiatric - illness/violence framing, quantitative evidence on suicide-reporting contagion or population-level violence risk, or ethical governance of commentary on individuals' psychiatric status once part of a legal record.

Results: Eighteen sources - peer-reviewed studies, international guidelines, and national ethical/regulatory documents - were synthesized. Suicide-reporting governance was found comparatively well-developed: a 2020 systematic review and meta-analysis confirmed a consistent association between the volume and detail of non-fictional suicide reporting and subsequent suicide rates. Governance of psychiatric - illness/crime framing was markedly weaker: a 20-year US content analysis found 55% of mental-illness news stories centred on violence, with mass-shooting framing more than doubling over the study period - a disproportion underscored by the landmark MacArthur Violence Risk Assessment Study, which found that discharged psychiatric patients without a co-occurring substance use disorder showed rates of community violence not significantly different from other residents of the same neighborhoods. This failure recurs internationally: German print coverage of the 2015 Germanwings crash attributed the co-pilot's actions to his psychiatric history in 64.1% of articles, with explicit stigmatizing content in 31.5% of texts, despite an unproven causal link between depression and violence against others . Citizen-journalist coverage by people with lived experience showed markedly less violence framing than professional coverage of the same topic (23% vs. over 40%) , and this framing effect extends directly into audience response: a content analysis of 558 online schizophrenia-related news articles found that stories framing the illness around violence or crime elicited stigmatizing reader comments, while stigma-challenging stories elicited stigma-challenging ones. Governance specific to public commentary on individuals' psychiatric or psychological status during active legal proceedings was found substantially underdeveloped regardless of the individual's legal role or whether any diagnosis was ever confirmed: a 2026 cross-national audit of all 145 WPA Member Societies found only 13 (8.96%) held any ethical provision addressing press or public commentary, and 83 (57.24%) had none at all; existing forensic-ethics literature addresses confidentiality within the evaluator–evaluee relationship itself - for instance, that an evaluee must be informed that standard confidentiality does not extend to the judicial authority - but does not address unconfirmed public speculation of this kind at all. The North Macedonian case illustrates this directly: the national Media Ethics Council has previously sanctioned unethical suicide reporting, yet no comparable standard existed once speculation about the same kind of psychological material resurfaced in the context of a criminal trial.

Conclusions: Governance of psychiatric and psychological commentary during active legal proceedings lags substantially behind other, well-evidenced areas of media–mental-health ethics - a gap the Germanwings and North Macedonian cases each illustrate independently, over a decade apart, and one that applies whether or not any diagnosis is ever clinically confirmed. We recommend forensic-specific reporting guidance jointly developed by national psychiatric associations and media-ethics bodies, coordinated internationally through the WPA's ethics section.

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